Medicine (402 questions) - Factual Memorization Verified

Built from the original source. Fact cue + answer for all 402 questions in Medicine (402 questions).

Dermatology (59 questions) (59 questions)

#	Fact cue	Answer
1	False statement about acne rosacea	It can present in one type of its clinical variant
2	A case typical of acne rosacea. Which of the following is false?	Hydroquinone 2-4% with or without oral erythromycin is the treatment
3	A case of greasy rash that worsens in the winter and has dandruff. Which of the following areas do you not need to examine?	Antecubital fossa
4	An obese female presented with a red, moist rash which was surrounded by pustules and papules, under both breasts. Which of the following is incorrect?	Oral antifungal, fluconazole, is needed in all of patients
5	Child presents with swelling of the lips with some difficulty breathing but is otherwise well and has no other rash. Which of the following is not used for treatment?	topical steroids
6	all are benign skin tumor except	Actinic keratosis
7	Patient complains of white fine scaling from the scalp which is mildly itchy, worsens in the winter. The patient was diagnosed with dandruff. Which of the following conditions is this related to?	seborrheic dermatitis
8	A typical case of erythrasma with brown scaly lesion in axillary area and ....., what's the causative organism	Corynebacterium minutissimum
9	Incorrect about pityriasis rosea	Has a chronic and remitting course
10	A patient presented with erythematous papules and plaques that were pink in colour and scaly. The lesions were on the trunk, back, upper extremity and lower extremity. The doctor peeled off the scales and the lesion began to bleed. Which of the following drugs is used for treatment?	Methotrexate tab
11	A case was given. Which of the following is true regarding psoriasis?	fine silvery scales
12	False regarding atopic dermatitis	There is flexiral involvement in infants
13	A case of a man with an ulcer (primary syphyllis). How to diagnose?	Microscopy from sample of ulcer
14	A 7-year-old child presents with itchy papules and vesicles, especially at night, on the web spaces of the fingers, wrists, and abdomen. Several family members are experiencing similar symptoms. The child was treated with topical betamethasone, but symptoms did not improve. What is the next best step in management?	Permethrin cream %5
15	A 72 year old man presented with painful vesicles along one side of chest, history of sensation of tingling and burning in the same area that preceded the rash, diagnosed as shingles which one is wrong	Treatment of the symptoms with antihistamine and calamine lotion will be enough
16	A 19-year-old female presents with inflammatory papules, pustules, and comedones on her face and upper back. She has a history of oily skin and worsening acne before menstruation. Based on the clinical presentation, she is diagnosed with Acne Vulgaris. which one is not true?	The bacteria involved in acne is a Gram-negative anaerobic diphtheroid.
17	A 17-year-old male presents with severe, nodulocystic acne on his face, chest, and back. He has tried multiple topical treatments with minimal improvement. On examination, he has multiple inflamed nodules, cysts, and scarring. He reports significant psychological distress due to his appearance.	Oral isotretinoin therapy

#	Fact cue	Answer
18	An irritable infant that's crying at night and doesn't sleep well has papules, nodules and vesicles on the palms, soles and face. What's the diagnosis	scabies
19	Patient complains of itching, diffuse redness and plaques and burning sensation with fine scaling of the scalp. She reported recently using henna before occurrence of symptoms. What's the diagnosis + treatment?	Allergic contact dermatitis + betnovate lotion
20	A 27-year-old man accidentally had sexual contact with a female totally unintentionally without meaning (like completely by accident, he promises), he developed discharge and soaked his underwear. He received treatment. Which of the following microorganisms may usually have a co-infection with this microorganism?	chlamydia
21	Incorrect statement about the patient presents with thickening and yellowish discoloration of the index and thumb nails	Redness and swelling of the proximal nail fold is caused by dermatophyte infection
22	Patient complains of white fine scaling from the scalp which is mildly itchy, worsens in the winter. The patient was diagnosed with dandruff. Which of the following conditions is this related to?	Seborrhic dermatitis
23	73 year old patient complains of itching. He recently visited a village to see some relatives who also had itching. The differential diagnosis includes all but:	Vitiligo
24	A case of erythematous rash with white adherent scales in the intertriginous area. Which of the following is incorrect?	Itching if present is an important feature of flexural psoriasis
25	A female pateint developed very itchy round well circumcribed erythematous papule of her lower abdomen, become worse at night and she got married 4 months ago, and her husband has the same complaint but he has it about 6 months ago, which of the following is not true about the complication :	scabies increased risk of HIV
26	Female with thinning of hair in the vertex	Androgenic alopecia
27	Infant with moist crusted erythematous lesions on the cheeks. What's the diagnosis?	Atopic dermatitis, subacute eczema
28	Which is incorrect about melanocytic nevi	Compound nevi are flat macules
29	A 24-year-old male presents with hypopigmented and hyperpigmented scaly patches on his upper chest and back. He reports mild itching, especially in hot and humid weather. KOH (potassium hydroxide) examination reveals "spaghetti and meatballs" appearance, confirming the diagnosis of Tinea Versicolor (Pityriasis Versicolor).	Wood's lamp examination shows green fluorescence.
30	In pityriasis all are true except	Surface of lesion is covered by thick brown scale
31	Case of down syndrome and atopic dermatitis. He had patchy hairy loss.	Alopecia areata
32	A patient has wet oozing discoid eczema. Which of the following formulations is appropriate?	Betnovate solution
33	A child presents with clinical features consistent with alopecia areata (the actual diagnosis was not given, only clinical features were). The parents ask about the prognosis. All of the following are bad prognostic signs except: Alopecia universalis	Adult onset of alopecia areata
34	case of a mass on the lower cheek of an elderly male for 2 years with rolled edges, recurrent bleeding, telangiectasia.	BCC
35	The followings are side effects of isotretinoin except	Nail pitting
36	Doesn't heal by scar	Wheal
37	In vitiligo which statement is not true	On biopsy there is normal number of non-functioning melanocytes
38	Patient complains of itching, diffuse redness and plaques and burning sensation with fine scaling of the scalp. She reported recently using henna before occurrence of symptoms. What's the diagnosis + treatment?	Allergic contact dermatitis, betnovate lotiom

#	Fact cue	Answer
39	Which cell increased in allergy	Eosinophils
40	Choose the correct answer	Molluscum contagiosum produces umbilicated papule
41	Appropriate way of washing clothes after scabies is	Simpley wash them and discard for 3-7 days
42	A mother has a child with atopic dermatitis, she uses an over the counter antipruritic agent. She says her child is drowsy and his performance in school has worsened. generation...	1st generation, diphenhydramine
43	STD s doesn't cause vaginal discharge	HPV
44	cells is not found in the epidermis	fibroblast
45	A 25-year-old unmarried lady presented with patchy depigmentation following a car accident. All of the following is true about her condition except:	The differential diagnosis includes vitiligo and post-inflammatory hypopigmentation, and a skin biopsy is needed to differentiate them
46	findings is an objective indicator of severe itching	excoriation
47	A sibling attended an inpatient ward for her sister who was a patient there (immunocompromised). One day later, she developed chickenpox. What to do	Give zoster immunoglobulin to the immunocompromised
48	A patient had an ulcer on his genitals 3 months prior that healed on its own and is now presenting with focal hair loss, generalized rash of reddish-brown papules and plaques and lumps in the groin with a broad base. Which of the following is the best way to confirm the diagnosis of this case?	TPHA
49	A mother comes with a child who has been diagnosed with atopic dermatitis. topical steroids is suitable	Hydrocortisone, ointment
50	A case was given, and the diagnosis was made: candida intertrigo. Which of the following is incorrect?	Oral antifungal, fluconazole, is needed in all of these patients
51	A child has recently entered to the school. He is experiencing stress. Overall hair is generally normal. There is irregular patch of hair loss with short, broken hairs.	Trichotillomania
52	A case about a 4-month-old child with oozing and crusting over his face and extensor surfaces, diagnoses, and aggravating factors	Atopic dermatitis and wool clothing
53	skin lesion will heal with scar	Nodule
54	There was a question about erythematous papules and pustules after stopping steroids. Which one is incorrect	Generalized pustular psoriasis is the first provisional diagnosis
55	Patient develops fine dry scaling of the palm, more prominent in the palmar creases. The other hand doesnt have any findings. Which of the following does NOT apply?	Topical antifungals are used for treatment
56	A 14 years old boy with white heads, black heads, and pustules, his acne has been getting worse. What's the management	topical retinoid
57	A child complains of itching of the scalp. She has a relative that's also complaining of the same issue. What's the diagnosis	Pediculosis capitis.
58	A case of a red lesion with raised borders on the face systemically unwell patient	Erysipelas
59	What's the word prognostic feature in alopecia areata	Associated with atopic dermatitis

Endocrine+Infection (1 questions) (1 questions)

#	Fact cue	Answer
1	supports an ADRENAL cause of cushing syndrome	low ACTH

Family Medicine (93 questions) (93 questions)

#	Fact cue	Answer
1	A 51-year-old man reports previous problems with "high cholesterol" and wishes to have hischolesterol checked today. On examination, his blood pressure is 170/100 mm Hg at rest for 5minutes and his pulse is 84 beats per minute and regular. His body mass index is 31. His familyhistory is significant for hypertension in both parents. His uncle is deceased of a myocardialinfarction (MI) at the age of 51 years, and his older brother had "heart problems" at the age of 53years and is in "fair health." You order a fasting lipid panel that reveals a total cholesterol (TC),288 mg/dL; triglycerides, 262 mg/dL; high-density lipoprotein (HDL), 37 mg/dL; and low-densitylipoprotein (LDL), 199 mg/dL. Regarding this patient's lipid profile, which of the followingstatements is true?	intense dietary therapy and drug therapy should be considered immediately
2	A person has positive family history for colorectal cancer, when he should do screening	40 yo
3	duchene muscular dystrophy	X-linked recessive
4	Visceral pain is	vague
5	requires lipid lowering agent mostly	52 Y.O, male, smoker and DM with LDL more than 140 mg/dl
6	Which one should be used in end of life care	Open and honest discussion
7	For most of the patients target BP is	<140/90 mmhg
8	A 70-year-old patient with hypertension and a history of chronic kidney disease is started on antihypertensive medication. Which class of drugs is often preferred for patients with both hypertension and kidney disease due to their renal protective effects?	ACE inhibitors
9	The timed get up and go test (raise from chair, walking 10 feet, turning around, returning to the chair, turning and sitting back down the chair) Should be completed in how man...	Less than 10 seconds
10	a 3 year old child presents with a cough. His breathing rate was 34/min, he had no chest indrawing, wheeze, danger signs or hypoxia. What's the classification?	No pneumonia
11	A 40-year-old patient with hypertension and a history of migraines is being treated with anantihypertensive medication. Which class of antihypertensive drugs may be contraindicated in this patient due to its potential to worsen migraines?	Alpha-blockers
12	A 45-year-old patient presents with lower back pain, radiating down the back of the right leg, following a fall at work. Physical examination reveals a positive straight leg raise test.Which of the following conditions is the most likely diagnosis?	Lumbar herniated disc
13	A patient presents complaining of dizziness. On further questioning, the patient characterized the sensation as "spinning" and "falling forward". What does the patient have?	Vertigo
14	A 32 year 12 weeks gestational age was diagnosed with gonorrhea cervicitis. What's the treatment	Ceftriaxone
15	RANKL is expressed on	Osteoblasts
16	A 45-year-old patient presents with lower back pain, radiating down the back of the right leg, following a fall at work. Physical examination reveals a positive straight leg raise test.which of the following imaging studies should be done first in theevaluation of this patient?	No imaging indicated.
17	Risk of heart attack will reduce to 50% after how long from smoking cessation	1 year

#	Fact cue	Answer
18	An old woman diagnosed with osteoporosis by DEXA scan what will be the initial treatment	Bisphosphonate
19	A 29-year-old woman presents to the ED with sudden onset of a severe headache involving bilateral occipital pain associated with nause	Head CT
20	You're going to advise a woman about risks of smoking during pregnancy. which one is true	It causes preeclampsia
21	A 7 week old child presents with fever and poor feeding for 24 hours	Very severe disease or possible serious bacterial infection
22	A child with respiratory symptoms diagnosed as pneumonia. What is the first line treatment?	Oral amoxicillin or amoxiclav
23	vitamins is a well-established therapy for the treatment of nausea and vomiting in pregnancy	Vitamin B6
24	A 72 year old patient was discovered to have atrial fibrillation, and has no murmurs or other abnormal findings on examination. What investigations do you send for next?	Blood investigations, Echo
25	A 32-year-old woman comes to your office for a complete physical examination. When discussing her vaccinations, you discover that she received her primary tetanus series as a child, and her last tetanus booster was 11 years ago. Which of the following is true?	The patient should receive tetanus, diphtheria, and acellular pertussis (Tdap) immunization.
26	In a pregnant woman with no risk factors for diabetes, when do you screen for gestational diabetes mellitus	24-28 weeks
27	A case came with blood pressure 128/83, HDL 48, and cholesterol 210. It was aboutscreening:	Screening should be started for blood pressure every 3 years
28	An obese, diabetic and hypertensive patient on metformin and another medication is trying to control her weight but failing. What is the appropriate additional medication?	Liraglutide
29	You are caring for a newly established 54-year-old white male patient with T2DM, hypertension and dyslipidemia. He does not have any clinical ASCVD, but his father died from an...	Treatment with high intensity statin
30	Ambulatory BP monitoring is required for	All above (Options: A. Sustained HTN; B. Masked HTN; C. White coat HTN; D. B and C; E. All above)
31	A 42-year-old man who has been your patient for 4 years comes to the office for a routine office visit. You have been treating chronic migraine headaches for the patient. He mentions that he would like to see a neurologist who he heard speak on the radio. Although you have done a complete evaluation of his headaches, he is concerned that he might have a brain tumor or other disease,Considering the situation, what should you do?	empathize with the patient about his symptoms and refer him to the neurologist he mentions
32	A 30-year-old patient with hypertension is found to have elevated aldosterone levels. What condition should be considered as a potential cause of secondary hypertension in this patient?	Primary hyperaldosteronism (Conn's syndrome)
33	You're doing a screening physical examination for a 40- year-old female patient. She does not have diabetes or known coronary artery disease. You calculate her 10-year atheroscl...	Lifestyle modifications, such as diet and physical activity
34	A Mother of one day post-partum comes to you complaining that her child Isn't getting adequate milk and she has low amount of milk, what's the right thing to tell her	The first milk is colostrum and its small in amount
35	What is correct regarding DNR (Do not resuscitate)	to let natural death occur without CPR
36	True regarding delirium and dementia	Polypharmacy and multimorbidity may be responsible for delirium
37	Which one is correct about Zyban	Contraindicated in eating disorders
38	A 3 week old child presents with jaundice extending to his palms	Severe jaundice

#	Fact cue	Answer
39	An intrauterine pregnancy of approximately 10 weeks gestation is confirmed in a 30-year-old G5P4 woman with an IUD in place. The patient expresses a strong desire for the pregna...	Remove the IUD immediately.
40	A 60-year-old patient complains of chronic lower back pain that worsens with rest and improves with activity. Morning stiffness lasting more than 30 minutes is noted. Physical examination reveals limited lumbar spine mobility. Which of the following is the most likely diagnosis?	Ankylosing spondylitis
41	A 60-year-old patient with hypertension is started on a thiazide diuretic. What importantlaboratory parameter should be monitored regularly while on this medication?	Serum potassium levels
42	True regarding the feeding of a newborn	Breastfed newborns need to be supplemented with vitamin D drops
43	Niacin at the theraputic dose cause	increase HDL and decrease cholesterol
44	A 18 week GA pregnant lady comes for antenatal care. On the last visit, her BP was 150/90 mmHg with no proteinuria. On the current visit, her BP was 148/85 mmHg, she has no prot...	Macrosomia
45	What is the most correct nerve block for upper abdominal visceral pain	celiac plexus
46	After checking fasting blood sugar for women, it was 150 mg in two occasions and she has family history of diabetes type2. what will be the treatment	Therapeutic lifestyle change and metformin
47	Bone remodeling depends on	Serum calcium level, Parathyroid hormone and thyroid hormone
48	A 57-year-old man is reviewed in a hypertension clinic, where it is found that his blood pressure is 165/105 mmHg despite standard doses of amlodipine, perindopril, doxazosin and bendroflumethiazide. Electrolytes and physical examination have been, and remain, normal. Which of the following would be your next stage in his management?	Arrange for his medication to be given under direct observation
49	A 4 month child is brought in for her vaccinations. She received her vaccinations at birth and at 2 months of age. All of the following are given at this visit except	IPV2
50	A 3- years old child with diarrhea, and there is cholera in your area. What is the first choice antibiotic	Erythromycin
51	An 8 week old child presents with cough. His respiratory rate was 63 breaths/min. No other finidings	Pneumonia
52	statements regarding type 2 DM is false	elevated fasting blood glucose concentration provides the first evidence of loss of glucose control
53	According to MUST all are correct except	more than 3 days without feeding and ill patient is at risk of malnutrition
54	A 49-year-old man has recently been diagnosed with type 2 diabetes and is being carefully monitored. He has been advised to maintain a healthier diet and lifestyle, he attends a follow-up clinic and claims to have been following the diet stringently since his last appointment three months ago. The most appropriate investigation is:	glycated hemoglobin
55	You have been caring for 36-year-old man and identified that he meets criteria for therapeutic lifestyle changes (TLC) to improve his profile after four months of adhering to yo...	Treatment with a low intensity statin
56	A young man complained of whitish penile discharge, dysuria and on examination there was redness of the penile meatus, and discharge stained his underwear. He has multiple sexual partners. Which of the following is the appropriate treatment?	Azithromycin
57	is not checked in a 6 week old child	Throat problems
58	A contraindication for OCP	Stroke

#	Fact cue	Answer
59	80 year old man with fatigue, anorexia and palpitations. He also had loss of vibration, sensation and proprioception of lower limbs what investigations is most appropriate to send?	Serum B12
60	A 32 year old female patient complaining of dizziness presented to the outpatient care center. She says this isn't the first time she's felt like this. She has been feeling this for a while, when she gets out of bed or turns in bed, after a few seconds lasting for a few minutes. This time, it lasted a few hours. What is the first line treatment?	Physical therapies directed at the disease source
61	A 24-year-old woman presents for an initial prenatal visit. She is at 9 weeks' gestation based on her LMP but, on further questioning, she is not certain of the first day of her LMP. Which of the following would be the most accurate estimate of her gestational age?	A first-trimester ultrasound
62	Which one is true	Vitamin D drops are given to newborns
63	A 45-year-old patient with hypertension is noted to have a blood pressure discrepancybetween the arms. The blood pressure in the right arm is significantly higher than in the left arm. What condition should be suspected in this patient?	Coarctation of the aorta
64	In smoking cessation, the 5 As stand for all except	Assess their mental state
65	Which an)hypertensive ABSOLUTELY Contraindicated in pregnancy	Lisinopril
66	A 28-year-old with a history of epilepsy presents for a preconception consultation visit. Which of the following is the most important advice to give to this patient?	Preconception folate supplementation
67	) is the best walking aid for someone who had total spinal injury at the level of the T10	wheelchair
68	You are discussing cancer screening with a patient. Her father was diagnosed with colorectal cancer at age 72. When should you recommend she begins colorectal cancer screening	50 years
69	Case seeking family planning with many unnecessary details, finally was recommended progestin only pill (POP). Which of the following is true about POP?	Acne may worsen
70	A 55-year-old patient with a history of chronic lower back pain presents with sudden, severe back pain after lifting a heavy object. Physical examination reveals tenderness over the spinous processes of the lumbar vertebrae and pain with extension and lateral bending.Which of the following is the most likely diagnosis?	Vertebral compression fracture
71	A 62-year-old male with hypertension and metabolic syndrome sees you for follow-up. A fasting triglyceride level is 300 mg/dL. You address lifestyle and other potential causes of his elevated triglycerides, including his current medications. If included in his current regimen, which one of the following hypertension medications would be most likely to contribute to his hypertriglyceridemia?	Metoprolol
72	The treatment for acute labyrinthitis is	Rest and antiemetics
73	features does not indicate a simple mechanical back pain	Age>50
74	A Case with palpitation, weight loss, and sweating. Investigations: TSH 0.2 (low), T4 normal, T3 9.9 (elevated), whats the next step investigation?	Radio-isotope uptake
75	is not true about patient care	The more people managing a patient the better
76	A middle-age man works at office. His blood pressure was 160/100 mmHg, which one is not true	One reading of 100 mmHg diastolic is diagnostic
77	Man and wife came for clinic, he was posi)ve for HBV, whats the CORRECT Statement for this NEWLY diagnosed case	all of above (Options: A. all family members should be screened for HBV; B. he should be told about the disease and its route of spread; C. HBV DNA is present in all body fluids; D. all of above; E. ,,)

#	Fact cue	Answer
78	A 58-year-old white woman presents complaining of low back pain for exactly 1 month after a fall. She has no history of fever, unexplained weight loss, diabetes, or cancer. Her pastmedical history is significant for mild persistent asthma and nicotine dependence. She had ahysterectomy for uterine fibroids at age 40. Which of the following characteristics should prompt further evaluation of her pain?	History of corticosteroid use
79	Which one is not true about falling in elderly	Functional impairment has less effect on risk of falling
80	65 year old patient with history of IBS with alternating constipation and diarrhea, recently has had a worsening of his symptoms. He hasnt had a colonoscopy before. FOBT was negative. What's the management?	Colonoscopy
81	A 23-year-old woman presents for her postpartum visit and contraception management. She delivered by spontaneous vaginal delivery 6 weeks ago and is breastfeeding. After reviewing her history and performing physical examination, you discuss the various methods of contracep■on with the patient. She opts for depot medroxyprogesterone acetate (DepoProver	Prolonged anovulation
82	in the therapeutic cascade,if the patient develop side effects after use of medication what we will do	we change the original drug
83	A 12-month-old child comes for vaccination for the first time and has not received any vaccines. is not given	BCG
84	A Case of anterior MI 4 months ago, BP checked and was 135/80 mmHg, On simvastatin, Aspirin, and metoprolol. LDL few days ago measured and was 70mg/dl. Whats the next plan?	Increase dose of simvastatin
85	Yoh are going to advise a woman about risks of smoking on pregnancy. which one is true	It causes preterm labor
86	A 33-year-old man weighs 159 kg at a height of 1.78 m. He has lost up to 45 kg on various diets but has always regained his lost weight after 1 to 2 years. He has been morbidly obese since he was 25 years old. He wishes to discuss bariatric surgery. You reply that each of the following is true, except for which one?	Indications for bariatric surgery include osteoporosis
87	Among the factors that should be considered in choosing a consultant are: Previous experience and communication with the consultant	True
88	is correct regarding the amount and frequency of infant feeding in a 10-month-infant	1⁄2 cup of 3-4 main meals + 1-2 snacks
89	statement is true about hydronephosis or Hydroureter during Pregnancy	May be dilated as a result of progesterone effects and mechanical compression of the ureters at the pelvic brim
90	In couple with secondary infertility for 2 years	reversible conditions usually the cause
91	communication is a one-way street	False
92	Emotional causes make up around 40-80% of causes of fatigue	True
93	45 years old male presented to CCU with severe chest pain he has history of taken viagra (sildenafil) 50 mg before 6 hours	avoid using nitrate in management for 20 hours

Geriatrics (1 questions) (1 questions)

#	Fact cue	Answer
1	For measurements of height the following steps are true	At birth is about 50 Cm

Hematology (72 questions) (72 questions)

#	Fact cue	Answer
1	is not mandetary to be screened in donorblood	Typhoid
2	Macrocytic RBCs can't be present in	IDA
3	Which of these doesn't happen in multiple myeloma	Splenomegaly
4	A typical case of IDA 65 years old, cases required	Chronic Gi bleeding
5	is true of both Factor VIII and vWF	Both are stored in Weibel-Palade bodies in endothelial cells.
6	A case suggesting leukemia , also having blast cells and features of anemia , WBC 50, which one can happen	Anemia , leukocytosis , thrombocytopenia
7	Incorrect regarding treatment in haematology	Fludarabine, cyclophosphamide and rituximab (FCR) for CLL in age more than 65 years old
8	A female patient presented with confusion and fever. She had a creatinine level of 4.8, and a low platelet. She had no other abnormal findings. What is the most likely diagnosis?	Thrombotic Thrombocytopenic Purpura
9	intra articular bleeding can occur in	VWD 2-N
10	An old age man presented with dementia and memory loss, having tingling sensation ad parastheia on the foots, features of pallor and anemia	B12 defiency
11	Autologous BMT done in the following except	Aplastic anemia
12	A 10-year-old boy presents with fatigue, jaundice, and mild abdominal pain. His mother mentions he has looked yellow on and off since infancy. He has a history of anemia requiring blood transfusion during a viral illness. On examination, he has scleral icterus, her sister had splenectomy at 12 years, Whats the underlying cause/mechanism of this?	Alterations of vertical interactions between the cell membrane and skeletal proteins.
13	Alcoholic patient with a poor diet develops megaloblastic anemia in the absence of neurological signs. deficiencies is responsible	Folate
14	DIC is characterized by all, except	Thrombosis more common in acute DIC
15	False regarding large b-cell lymphoma	In stage 1 & 2 you may manage conservatively
16	A 15-year-old boy presented to you complaining of having recurrent attacks of fatigue and red urine, especially during stress and the spring season. He had Hb electrophoresis done at 3 months of age, which showed HBF 20%. Which of the following is the most probable diagnosis?	G6PD deficiency
17	5 years old female sickle cell case, had packed red cell transfusion (10ml/kg) every 3-4 weeks to prevent stroke, after 10 mins of transfusion she told her mom " feel yucky...	Acute hemolytic reaction
18	is wrong about AML-M3	It needs chemotherapy followed by bone marrow transplantation
19	Which one is wrong about follicular lymphoma	it is aggressive tumor
20	ALL is characterized by all the following, except: a. CNS involvement b. Lymphadenopathy c. Testicular infiltration d. Mediastinal mass e. Gum hypertrophy	Gum hypertrophy
21	suggests TTP	Schistocytes in peripheral blood smear
22	A 10-year-old boy presents with fatigue, jaundice, and mild abdominal pain. His mother mentions he has looked "yellow" on and off since infancy. He has a history of anemia requiring blood transfusion during a viral illness. On examination, he has scleral icterus, her sister had splenectomy at 12 years, Whats the underlying cause/mechanism of this?	Alterations of vertical interactions between the cell membrane and skeletal proteins.

#	Fact cue	Answer
23	In which of following organs iron will not deposits	brain
24	Severe hemophilia A	Factor VIII activity <1%
25	Typical case of multiple myeloma with low back pain and renal failure and features of anemia	Multiple myeloma
26	A case of an old male with controlled hypertension, no other signs and symptoms platelet count of 650,000/ml. JAK 2 +ve; treatment?	Observation only
27	30 year old female patient complains of recurrent thrombosis and miscarriages. Her coagulation profile was significant for PTT prolongation that did not correct with mixing.	Antiphopholipid syndrome
28	A patient with skin bruising and prolonged bleeding time, can be due to all the following, except:	Vitamin K deficiency
29	In cases giving an)coagulants is not recommended	Pulmonary hypertension
30	Sickle cell case, receiving transfusion of PRBC every 3-4 weeks, Suddenly feels (yucky), fever and back pain, whats the diagnosis	acute hemolytic transfusion reaction
31	A 70 year old female patient presents with fever and night sweats. She has palpable nodes in her neck. Her CBP showed elevated WBC count and smudge cells. Flow cytometry revealed CD23 and CD5 positive cells.	CLL
32	Alcoholic patient with a poor diet develops megaloblastic anemia in the absence of neurological signs. deficiencies is responsible	Folate
33	A case of lymphoma with a mass in the mediastinum and CT for chest abdomen and pelvis found nothing else. What is the stage?	Stage 1
34	does NOT explain the pathophysiology of ITP	Hypersplenism
35	which one doesn't cause fragmentation anemia	Burn
36	A 68-year-old man who presented with severe anaemia and progressive splenomegaly to the ED with small bowel infarction with a Hb count of 7mg/dL, MCV of 90, Plt of 70, and WBC of 30,000. A peripheral blood smear showed 60% neutrophil, 15% metamyelocyte, some myelocyte, 8% basophil, 4% eosinophil and ? % ... There were also misshaped RBCs and most of them showed teardrop appearance with some nucleated RBCs.	Primary Myelofibrosis
37	is molecular diagnosis of hereditary spherocytosis	Weakened vertical interaction between proteins and cell membrane
38	A patient with B thalassemia minor is	Folic acid is mandatory for all.
39	Case of lymphoma. Which of the following is wrong? a. B cells indicate a good prognosis	B cells indicate a good prognosis
40	is found in normal adult hemoglobin	A, A2, F
41	Respiratory assessment is required in anticoagulants	Ticagrelor
42	A 65 year old patient has a CBC showing a WBC count of 35,000. 80% are lymphocytes. She has no other symptoms, no lymph node enlargement or hepatosplenomegaly. What's the next best test?	Flow Cytometry
43	vWD is characterized by all, except	joint deformity
44	Which one happens in Megaloblastic anemia	Normal serum folate but decreased RBC folate
45	All of the following presents with mediastinal widening on CXR, except	Multiple myeloma

#	Fact cue	Answer
46	A 20-year-old man with known history of hemophilia. You discuss his condition with him. Which of the following is WRONG regarding haemophilia?	PT and INR will be elevated.
47	Mr. Sam presents with no previous medical disease or medications and with macrocytic anemia (Hb = low and PLT = low and WBC = low). His reticulocyte count was 0.2% (low). invest...	Bone marrow study
48	All are clinical features of DIC EXCEPT	tear drops On blood film
49	Iron deficiency anaemia is of concern in	All of the above (Options: A. Pregnancy; B. Old age; C. Cardiac patients; D. Children; E. All of the above)
50	bone in adults used for bone marrow biopsy	Iliac bone
51	is NOT a line of Thalassemia treatment	supplemental iron
52	is not a cause of thrombocytopenia in idiopathic thrombocytopenic purpura (ITP)	Hypersplenism
53	A case of CML in an elderly female with CBC showing increased white blood cells with normal hemoglobin and platelets, with different stages of maturation, predominantly basophils. Which of the following is wrong?	No need for treatment at this stage
54	All of the following are characteristics of AML, except	CNS infiltration
55	platelets need respiratory symptoms monitoring	Ticagrelor
56	clinical situations represents a likely diagnosis of multiple myeloma	Acute renal failure in a 65 year old woman with hypercalcaemia, rib pain, immunoparesis for IgG and IgA but no intact paraprotein
57	A child presents with fatiguability and pallor. CBC showed Hb 8 mg/dL, PL T 35,000 and WBC 1,500. BM biopsy shows hypocellularity without blasts.	Aplastic anemia
58	JAK inhibitors can be given to	Only JAK2 positive myeloproliferative disorders
59	A 65-year-old man presented with 3 months history of left hypochondrial discomfort mild splenomegaly with cervical and axillary lymphadenopathy CBC showed Haemoglobin 9 WBC 140000 (60% lymphocyte) platelets 90000. Which of the following statements is wrong about this condition?	. Rai III stage
60	in very severe aplastic anemia ANC will be	less than 200
61	A case of a CML in the chronic phase, which one is wrong?	No need for treatment in this stage
62	A case of neck mass for 6 months 57years old with no blast cells , features of anemia mentioned ...best drug of Choice also high WBC 82 ?	FRC
63	A case of GVHD. Which of the following sites is least likely to be involved?	Heart
64	Dose of methotrexate in adult	7.5-25 mg/week
65	A case of PE with dyspnea of chest pain ..... The cause	Prothrombin gene analysis for the G20210A variant.
66	A Case with palpitation, weight loss, and sweating. Investigations were: TSH 0.2 (low), T4 normal, T3 9.9 (elevated), whats the next step investigation?	radio-isotope uptake
67	is NOT true about CML	coombs test may be positive
68	In Guillain-Barre syndrome the followings are regarded true, EXCEPT	Respiratory failure occur in almost all cases
69	All of the following is true concerning feeding recommendation in children except	The legumes should be started after the age of 8 months

#	Fact cue	Answer
70	All the following are true except	In Lambert- Eaton syndrome, a decremental response occurs with rapid stimulation.
71	true about urothelial bladder cancer	The diagnosis and initial staging of bladder cancer is made by cystoscopy and transurethral resection (TUR) .
72	All of the following is false regarding a follow-up visit in children under 5 years of age except	Pneumonia in 3 days

Nephrology (86 questions) (86 questions)

#	Fact cue	Answer
1	In which patient asymptomatic bacteriuria may present	Pregnancy
2	needs prophylactic antibiotics	Grade 4 vesicoureteral reflux (VUR)
3	Acute hyperkalemia is associated with electrocardiographic changes	QRS widening
4	After giving ACE and ARB, investigations do you follow	both A+B
5	False regarding nephrotic syndrome	Urine protein less than 3.5mg/dl
6	All of the following statements regarding the anemia of chronic kidney disease are true, except	Ferritin is reduced.
7	A child develops periorbital puffiness. He had a sore throat that resolved 2 weeks ago. His urinalysis showed proteinuria, hematuria and RBC casts. His ASO titer was elevated.	post streptococcus glomerulonephritis
8	Which type of renal tubular acidosis is associated with stone formation	Type 1
9	What is the commonest causative microorganism of UTI	E. Coli
10	Nephrotic syndrome is characterized by ? ●	Hyperlipidemia ●
11	Sterile pyuria can occur in the following conditions, except	Renal tuberculosis
12	Patient developed sepsis. He has renal impairment. Which antibiotic requires a major reduction in its dose?	Gentamicin
13	Most common cause of Chronic kidney disease in developed countries is	Diabetes
14	is FALSE about Renovascular hypertension	atherosclerosis usually affects younger man and fibromuscular dysplasia affect women over the age of 50
15	A 14 year old child presents with red urine. He's otherwise asymptomatic and normal on examination. What's the next step?	urine dipstick
16	is suggestive of complicated urinary tract infection	recent use of indwelling urinary catheter
17	Urine dipstick revealed 3+ albumin, 3+ blood, and RBC	Postinfectious glomerulonephritis
18	The diagnosis of polycystic kidney disease in a patient with positive family history requires	Ultrasound
19	case of a middle aged man with obvious renovascular hypertension) ... Which of the following drug should be given with caution in the context of this case?	Lisinopril
20	Problems with hemodialysis may include all the following except	Hypertension
21	One of the following is uncomplicated UTI	UTI in premenopausal women
22	Polyuria is NOT a feature of	Hypocalcemia

#	Fact cue	Answer
23	conditions associated with renal cysts is NOT premalignant	Autosomal dominant polycystic kidney disease
24	A child had diarrhea and now became unwell with pallor and delayed capillary refill. Multiple dipstick showed ++ RBC with ++ Protein.	CBC and blood film
25	is a hormone responsible for red blood cell reproduction	Erythropoietin
26	is not a cause of hyperkalaema	Insulin therapy
27	Which drug causes chronic kidney disease	lithium
28	is not a side effect of dialysis	hypertension
29	A patient has periorbital edema, and a urine dipstick showed a proteinuria of (+3). Best next investigation.	Serum Albumin
30	The following are typical features of the haemolytic uraemic syndrome, except	Thrombocytosis
31	does not cause dysuria	Renal cell carcinoma
32	23-year-old sexually active female had dysuria, lower abdominal pain, discharge. How can you differentiate between STDs and UTI?	Profuse vaginal discharge and itching
33	An 8-year-old presented with dysuria... (features of cystitis). Leukocyte +2 and Nitrites	Urine microsocpy and Culture
34	A 2 month old child has generalized swelling. His albumin is 1.5 g/dL	Congenital nephrotic syndrome
35	a case of CKD became uremic present with	pericarditis
36	is NOT a side effect of erythropoe)n therapy	hypotension
37	Nephrotic syndrome is characterized by	Hyperlipidemia
38	A pregnant woman of 4 months was screened and found to have symptomatic bacteruria of E.Coli type, whats the most appropriate next step	Nitrofurantoin
39	All the following is a function of the tubulointerstitial compartment of the kidneys EXCEPT	Ultrafiltration of the blood coming to the kidneys
40	Significant pus-cell count for UTI	10
41	does not cause transient hematuria	Essential hypertension
42	suggests glomerulonephritis	Protein and RBC cast
43	Urine dipstick revealed 3+ albumin, 3+ blood, and RBC	Postinfectious glomerulonephritis
44	conditions is NOT part of complicated UTI	Perimenopausal and non-pregnant
45	On urineanalysis if leukoesterase is posistive, what does that mean	Presence of WBC
46	Polyuria and polydipsia are features of	tubular disease
47	Significant pus-cell count for UTI	10
48	is not a cause of papillary necrosis	Syphilis
49	A long case. investigations is to be routinely followed when treating a case with renal artery stenosis with ACEI or ARB	creatinine and potassium
50	A child has generalized swelling 4 weeks a er starting prednisone.	Steroid resistant nephrotic syndrome
51	is not a complication of CKD	diabetes mellitus

#	Fact cue	Answer
52	A pregnant patient has complicated UTI, the culture shows gram negative cocci, what is the treatment	Nitrofurantoin
53	A 35 year-old man presents with autosomal dominant polycystic kidney disease. Which of the following is not an extra renal manifestation of this condition	Hepatic hemangioma
54	isn't an extrarenal manifestation of PCKD	Renal cell carcinoma
55	An 81 -year-old man admitted with a stroke becomes increasingly drowsy after receiving nasogastric feeding for five days. Which biochemical abnormality is the most likely cause...	Hypernatremia
56	A petient present with hyperkalemia, which is the urgent way to decrease pottasium level	Insulin and glucose
57	All of the following are a complication of nephrotic syndrome EXCEPT	Hypocalcemia
58	A Urine dipstick postive for blood could be caused by	Exercise
59	he most common presentation of Renal cell carcinoma is	accidental diagnosis
60	is wrong about renal cell carcinoma	Biopsy is indicated in all types of renal tumors
61	Which one is correct about acute interstitial nephritis	None of the above (Options: A. There is always eosinophilia; B. There is always eosinophiluria; C. Biopsy is always needed; D. There is always rash; E. None of the above)
62	is NOT a feature of nephritis syndrome	Hyperlipidemia
63	A 30-year-old man is found to have macroscopic haematuria, proteinuria of 1.5 g/24 hours and a serum creatinine level of 153 micro mol/l. What is the most likely diagnosis? * Diabetic nephropathy * Membranous glomerulonephritis * Minimal change nephropathy * Focal segmental glomerulosclerosis * IgA nephropathy	Focal segmental glomerulosclerosis
64	is TRUE about diabetic nephropathy	it's the most common cause of end stage kidney disease worldwide
65	All of the following are a complication of hemodialysis EXCEPT	Hyperglycemia
66	Idiopathic membranous nephropathy is associated with	Antibodies to the M-type phospholipase A2 receptor.
67	is wrong regarding anemia in CKD	anemia in ckd is well tolerated because of increase in 2,3DPG
68	what is the best way to prevent of progression of chronic kidney disease	Glycemic control
69	is not a side effect of erythropoietin injection	thrombocytopenia
70	Best test for diabetic nephropathy screening	Urine albumin
71	False regarding stages of renal disease of diabetic nephropathy	Stage V: GFR <15 mL/min
72	55-year-old man with a history of CKD presents with muscle weakness, bone pain, and recurrent kidney stones. He had a calcium level of 12.2 mg/dL, and low parathyroid hormone level of 8 pg. What is the most likely diagnosis?	Vitamin D intoxication
73	A 45-year-old uremic lady started her chronic hemodialysis program. The leading cause of death in patients on hemodialysis program is? * Cardiovascular disease * Hyperkalemia * Infection * Malignancy * Uremia	45-year-old uremic lady started her chronic hemodialysis program. The leading cause of death in patients on hemodialysis program is?
74	A pregnant woman (4 months) was screened and found to have symptomatic bacteriuria. What's the most appropriate next step	Nitrofurantoin

#	Fact cue	Answer
75	regarding acute kidney injury correct answer is	it usually develop over days too weeks
76	One of the techniques to build better work relationships is development of a routine for managing conflict	True
77	According to the international classification of VUR reflux with gross dilation of ureter, pelvis, and calyces, loss of papillary impressions, and ureteral tortuosity, its grade	Grade V
78	In general, the left kidney is more commonly affected than the right kidney in PUJ Obstruction	True
79	the important point about acute pyelonephritis is that it affects the normal functioning unit of the kidney causing destruction of the glomeruli	False
80	is CT urography essential in diagnosis of blunt renal trauma	Yes
81	•A mother brought their child for his routine checkup. On observation the child can build a tower of 9 cubes, when you give him a book he can turn the pages singly. The mother t...	30
82	All are true about EEG in epilepsy except	Up to 5% of patients with proven epilepsy will have a normal routine EEG
83	All the following are true about renal cell cancer RCC except	The classical triad of gross hematuria, flank pain, and a palpable mass occurred in most of cases.
84	All the following are true about TURP except	TURP syndrome hypernatremic state due to absorption of the irrigating solution
85	Wedge Resection of the Posterior Vesical Neck causing Primary VUR	False
86	all cases of blunt renal trauma need surgical reconstruction	False

Rheumatology (90 questions) (90 questions)

#	Fact cue	Answer
1	is not correct regarding CRP and ESR	ESR is characteristically very low in sickle cell disease
2	Radiological findings in grade 2 ankylosing spondylitis: a. Pseudo-widening b. Bone erosion c. Narrowing and osteosclerosis	Narrowing and osteosclerosis
3	A case of ankylosing spondylitis (AS) of the spine not responding to NSAIDs, whats next?	TNF alpha blockers
4	A 60-year-old lady with a previous colles fracture presents to the department with another colles fracture. A bone scan (DEXA) was low and was consistent with a diagnosis of osteoporosis. What is the best treatment?	Bisphosphonate
5	What's correct about polymyositis	It commonly causes proximal muscle weakness
6	A young female (25-year-old) presenting with polyarthralgia of three weeks duration associated malar rash in the face with painful oral ulceration, photosensitivity, hair fall with minimal joint swelling. On Examination: moderate pain and stiffness with joint manipulation, which of the following would be the cause?	All (Options: A. UV light; B. Infectious trigger; C. Genetics; D. Environmental toxins: Silica exposure is a known risk factor for SLE induction; E. All)
7	Acute gout is not treated with	Allopurinol
8	Middle aged man who recently started on bendroflumethiazide for his hypertension, and recently went to a party where he drank a lot of wine and pork, presents with very severe p...	Gout
9	can cause Raynaud disease	Systemic sclerosis

#	Fact cue	Answer
10	A case with diagnosed systemic sclerosis presents with sudden onset leg edema, elevated creatinine. What is the treatment of choice?	Candesartan
11	In rheumatoid arthritis the test for TB is mandatory before starting which of the followings	infliximab.
12	Which of the followings is correct regarding RA	RA is: can involve multiple organs and is due to immune dysfunction
13	A 60-year-old man has a rash on his fingers and around his eyes. He is finding it difficult to stand from sitting and is losing weight.	dermatomyositis
14	A case of SLE presented with sudden chest pain, SOB, fever with diffuse saddle shaped ST segment elevation on ECG, the most probable Dx is :	Pericarditis
15	Lymphadenopathy is a feature of	Systemic JIA
16	A male patient was referred from an ophthalmologist presenting with oral and genital ulceration on rheumatological examination and diagnosed with uveitis. Which of the following is the most probable diagnosis?	Bèhcets disease
17	38-year-old man has extreme pain in his left foot. It has progressed rapidly over the past 48h such that he is now struggling to stand. He says he has suffered this type of pain before and confesses to regular IV drug use. His foot is swollen and very inflamed, with erythema spreading proximally from the toes to the medial malleolus.	Cellulitis
18	A patient, who is a known case of rheumatoid arthritis, presented with weight loss, recurrent infection, and skin bruising. On lab tests, there was low WBC and neutropenia. What could be the possible cause?	Side effect of glucocorticoid
19	A young 25-years old man presents with Fever, chills, malaise, involvement of predominantly lower extremity joints (knees, ankles), urethritis Examination: Mono-or oligo-arthritis, tenosynovitis (wrists, fingers, ankles, toes), pustular or vesiculo-pustular skin lesions and aspirate of joint stained with gram stain showed G negative diplococci	GonococcalArthritis
20	Incorrect about psoriatic arthropathy	Very commonly the skin manifestations and arthritis appear together
21	A 38-year-old female has been diagnosed with Ankylosing Spondylitis (AS). She is HLA-B27 positive and is well controlled with NSAIDs. Now she is worried about her 8-year-old son developing a similar condition. What do you advise her?	The chance of her son developing AS is 10%
22	A 38 year old woman had several episodes of transient ischemic attacks. She had constitutional symptoms of fever, anorexia, and malaise. She also has claudication on walking only a couple of hundred meters. Examination revealed evidence of aortic regurgitation. Arteriography revealed diffuse stenosis of the aorta and its main branches. ESR is markedly raised. What diagnosis fits best with this clinical picture?	Takayasu's disease
23	A 50 year old female with raynaud's phenomena and esophageal dysphagia her blood tests have revealed a raised creatinine 180 micromol/L and she is hypertensive with BP 170/85mmhg. Which one of the following agent is the best treatment for her blood pressure.	Ramipril
24	A male presented to the accident and emergency department with fever, lethargy and chills, as well as tender and swollen knees. What is the appropriate next step?	Synovial aspiration and blood culture
25	A 36 year old female patient complained of chest pain radiating to her left arm. On examination she had no pulse in her left arm. Which of the following is the management?	Steroid and DMARD
26	A 62 years old female pateint complain of inability to upstairs and has difficulty of (?) and on examination she has Weakness and proximal muscles wasting what is the most likel...	Polymyositis
27	Diagnosis in a known case of ra, presented with shortness of breath and chest pain, tachycardia, bp 80/70 and ecg change showed low voltage qrs	Cardiac tamponade

#	Fact cue	Answer
28	A case of a parent with Ankylosing spondylitis with a 10 year old son who's worried about the risk of her son having AS, what to tell her?	Less than 10% risk of getting AS
29	A 9-year-old child with pain and swollen fingers and wrist, Rheumatoid factor is -ve, what's the diagnosis?	JIA
30	Patient with gout also has PUD and HT most appropriate treatment	colchicine
31	A male patient with worsening back pain especially on lateral flexion & bending forward. X-ray normal. What is the treatment of choice?	Sulfasalazine
32	A female with swelling and pain of hand and foot joint for 2 weeks. What is least likely to be a cause:	Fibromyalgia
33	A 24-year-old female presented with joint swelling, proteinuria, and symptoms consistent with pulmonary manifestations. She has positive anti- DsDNA and positive ANA. What is the most common pulmonary manifestation?	Pleurisy
34	Patient presents with an acute attack of joint inflammation in the foot (I dont remember the details but the case was very typical for gout). He has hypertension. He's had recurrent episodes for a while. What do you manage the current acute attack with?	Colchicine
35	A woman whose mother had recently been diagnosed with osteoporosis, was afraid that she would develop it to and came to the PHC to ask for measure to help her prevent it. exerci...	Swimming
36	A 56-year-old woman consults her doctor with a 3-month history of weakness of her limbs. She is finding it increasingly difficult to climb the stairs and can no longer carry heavy bags of shopping. She has no pain, but feels very lethargic. Neurological examination of her limbs reveals normal tone and muscle bulk with no fasciculation. Proximal power is reduced at 3/5 in all four limbs. All reflexes are present and normal. Her plantar responses are flexor. Sensory examination is normal.	polymyositis
37	A child presents with fever, conjunctivitis, cervical lymphadenopathy, and some other features. What is the diagnosis?	Kawasaki's disease
38	Lymphadenopathy is a feature of	Reactive arthritis
39	Which one is important in the diagnosis of SLE	Autoimmune hemolytic anemia
40	An obese hypertensive male aged 50 years, consumes alcohol frequently, presented with sudden onset of right first metatarsophalangeal joint pain and swelling. On Examination: redness, warmth, swelling, and exquisite tenderness of the affected joint.	Acute gouty arthritis
41	A diagnosed case of SLE have arthralgia and mouth ulcer, no end organ damage. What is the management	Hydrochloroquine
42	First line of treatment in SLE is	Hydroxychloroquine
43	Which one inhibits osteoclasts only	Denosumab
44	A 48-year-old female presented with recurrent episodes of joint pain and swelling in hand and feet. In examination you found tophi near her hand and toes. What is the most probable diagnosis?	Gout
45	A 65-year-old male patient presented with a one week history of swelling of the right knee joint. Synovial aspiration revealed CPPD crystals. What is the most appropriate diagnosis?	Pseudogout
46	A 60-year-old man has a swollen painful right knee. He has chondrocalcinosis on an X-ray.	Pseudogout
47	76..A case with red swallon joint pain that he cannot move his leg and has fever, A needle aspiration of his joint shows wbc of 150,000	infectious arthritis.
48	A patient has RA. His risk of death is increased, mainly from?	Coronary artery disease

#	Fact cue	Answer
49	A patient presents with fever, recent onset joint swelling and pain and limitation ofmovement. How do you reach a diagnosis?	Joint aspiration and culture
50	A young female (25-year-old) presenting with polyarthralgia of three weeks duration associated malar rash in the face with painful oral ulceration, photosensitivity, hair fall with minimal joint swelling. On Examination: moderate pain and stiffness with joint manipulation.	Systemic Lupus Erythematosus
51	First-line treatment for sle	hydroxycholoroquine
52	A guy presents with acute pain and swelling in the big toe for 1 day duration. His ESR (80) was elevated with normal blood pressure. What is the appropriate statement	Give NSAIDs immediately
53	A 72 year old female with unilateral headache of 2 weeks and blurry vision of the past 3 days. She had other features suggesting systemic inflammation, and the presentation was very suggestive of giant cell arteritis (I don't remember the details of the case). What is the most appropriate next step?	Start corticosteroids
54	is not a feature of mixed connective tissue disease	Rapidly progressing GN
55	A patient known case of RA, presented with 6 weeks hx of paraesthesia and numbness in both hands, was severe that awakened patient from sleep, there was no other neurological deficits on examination however Phalen test was positive bilaterally, lab investigations showed (CBC ...., ESR...,), what is the next step in management?	EMG
56	~32 years Female patient has recurrent transient ischemic attack, and imaging shows aneurysms of arch of aorta and it is branches, what is the most likely diagnosis	Takayasu arteritis
57	A patient presented with shoulder girdle pain upon abduction. What is the most likely cause?	rotator cuff tear
58	Case that's suggestive of ankylosing spondylitis. Which of the following does not support a diagnosis of spondyloarthropathy (SpA)?	Rheumatoid factor positive
59	What test is particularly specific for SLE	Anti DsDNA Ab
60	A female came to the rheumatology department complaining of pain and stiffness of her knees for the past 7 days. She has recent travel history to outside the country. She had a bout of Diarrhoea recently and 2 previous Diarrhoea over the past year. Which of the following is not an appropriate next step?	Ciprofloxacin for 2 weeks
61	Cause in a known case of sle, presented with chest pain, ecg revealed diffuse st elevation	Pericarditis
62	A 26-year-old woman presented with malar rash, photosensitivity, and arthralgia. On examination, there are oral ulcers. Her urine dipstick shows hematuria and proteinuria. Given the suspected diagnosis, which antibody determination would be particularly specific for the disease?	Anti-dsDNA antibody
63	Patient presents with swelling and pain of the knee, wrists and hands (MCP and DIP), and nail features; when her nail varnish flakes off, she has nail pitting and features of nail lysis/separation. What's the likely diagnosis?	Psoriatic arthropathy
64	methotrexate dose in child	1 mg/kg max 50mg/week
65	A case of systemic sclerosis with high creatinine, which drug is used	ramipril
66	First-line treatment for sle	hydroxycholoroquine
67	A 25 years old with inflammatory back pain worse at night and early in the morning and associated with morning stiffness, improved by movement. The patient has become impaired e...	MRI of the sacroiliac joint
68	A case of a parent with Ankylosing spondylitis with a 10 year old son who's worried about the risk of her son having AS, what to tell her?	Less than 10% risk of getting AS

#	Fact cue	Answer
69	78-year-old woman has noticed her left knee become increasingly swollen over the last 2 weeks. She has had pain in the joint for many years now. She takes alendronic acid 70mg PO weekly and calcium 500mg/colecalciferol 5 micrograms tablets PO at night. The joint is swollen and fluctuant with a positive bulge test. It is held in fixed flexion at 20°.	OA
70	A 59-year-old woman presents with symptoms of Raynaud's phenomenon and dysphagia. On examination, she has painful lesions on her fingers and facial telangiectasia.	Progressive systemic sclerosis
71	Patient presents with back pain, morning stiffness, sacroiliac joint tenderness, decreased lumbar spine motion. What's the diagnosis?	Ankylosing spondylitis
72	True about systemic sclerosis	Raynaud Phenomenon
73	Uveitis frequently seen in	Oligoarticular JIA
74	30-year-old healthy female presented with a 3 months history of changing color of the bilateral digit tips starting from pale to blue and then to red upon exposure to cold or stress, with a negative past medical history. Describe her medical condition:	Raynaud's disease
75	A case has bluish discoloration of fingers when exposed to cold temperatures, and hypertension 170/100 (?), and also hardening of skin of hands, had esophageal problem that not resolved after taking a course of PPI, what drug is prefered for controling her blood pressure?	Amlodipine
76	75.a child case complain of right knee pain, previously he got right ankle pain and swelling then when his ankle pain resolved, his right knee pain started, Diagnosis	Rheumatic fever
77	Dose of methotrexate in adult	7.5-25 mg/week
78	a case with painful Red swollen tender joint next step will be:	arthrocentesis
79	is not associated with pseudogout	Sarcoidosis
80	A 76-year-old man has a severely painful left ankle. It has come on rapidly over the last 3 days such that he can no longer put weight on it. He has hypertension and says he has been started on a new tablet within the last 3 months. His foot is very swollen and erythematous	Gout
81	A 60-year-old lady has painful joints, no swelling, normal ESR and normal CRP .	OA
82	A 63-year-old woman with a past medical history of systemic sclerosis presented with sudden onset of swelling hands and feet, laboratory investigations revealed a serum creatinine 2.1 mg/dL (normal range: 0.6-1.1 mg/dL). What is the most appropriate first line treatment?	captopril
83	Which organ seldomly involved in polyarteritis nodosa	Lung
84	A 72-year-old woman presents with a two-month history of headache, scalp tenderness, and jaw pain while chewing (jaw claudication). She also reports fatigue, weight loss, and low-grade fever. Recently, she has noticed blurred vision in her right eye. On examination: Tenderness over the right temporal artery with reduced pulsation. No focal neurological deficits. Fundoscopic exam shows anterior ischemic optic neuropathy (AION) in the right eye.	Giant cell arthritis
85	A patient develops sudden severe pain, swelling, warmth in his knee joint.	Joint aspiration and culture and sensitivity
86	drugs cause renal crisis in systemic sclerosis	Prednisolone
87	Which one is NOT a risk factor for osteoporosis	Overweight
88	Creatine kinase is not elevated in	Polymyalgia rheumatica

#	Fact cue	Answer
89	A 10 year old girl presented with a one week history of malaise, joint pain, and pyrexia. On examination there was evidence of a swollen erythematous knee. Her ankle had been similar a few days prior but this had now settled. What is the most likely diagnosis?	rheumatic fever
90	years old female diagnosed with SLE developed Renal involvement causing proliferative Glomerulonephritis, what's the treatment used	IV cyclophosphamide+ Prednisone

Neuropsychiatry (167 questions) - Factual Memorization Verified

Built from the original source. Fact cue + answer for all 167 questions in Neuropsychiatry (167 questions).

Neurology (71 questions) (71 questions)

#	Fact cue	Answer
1	A patient presents with right-sided weakness and was sent for a CT-Scan, which shows a Haemorrhagic Stroke. He had a blood pressure of 200/120 mmH	Labetalol
2	A patient with diplopia and a 6/6 visual acuity. After watching TV, his diplopia would worsen and he would develop ptosis.	Purely ocular myasthenia gravis
3	What is the power level in a patient who could only move his limb with gravity eliminated	2
4	A case has vertigo, ataxia, and perioral and le facial numbness. Which of the following is the most likely cause?	BS ischemia
5	is the empirical treatment of immunocompromised patients with bacterial meningitis	Ampicillin and Meropenem and Vancomycin
6	A man presented with shaking of both hands associated with stiffness. Examination revealed loss of swinging movement of the right leg. Which medication is NOT a line of therapy...	Olanzapine
7	In is anticoagulant not indicated	Pulmonary Hypertension
8	Regarding thunderclap headaches	Subarachnoid hemorrhage is the most common secondary cause and should be excluded
9	best describes the epidemiology of myasthenia gravis (MG), in terms of its incidence, prevalence, and populations at highest risk of developing this disorder	Peak incidence is bimodal, with highest risk in younger women and older men
10	A known case of uncontrolled diabetes and hypertension presents to the neurological department with right-side facial weakness and right arm weakness. The power of the lower lim...	Left superior branch of the middle cerebral artery
11	A patient with right sided weakness in the lower limbs. On MRI, it was diagnosed with ischaemic stroke. Which artery is most likely to be involved?	left anterior artery
12	True regarding corneal reflex	The efferent pathway is the facial nerve
13	Women who carry the mutation for Duchenne muscular dystrophy on one of their X chromosomes have an increased risk of	cardiomyopathy
14	The temperature & pin sense loss usually develops with disease in	posterior horns of spinal cord
15	MS medications can cause PML	natalizumab
16	Case of a patient with headache, neck s3ffness, and temp 37, what's the most appropriate dx?	Subarachnoid hemorrhage
17	A patient with worsening headache and vomiting had the following lab results. Opening pressure 300 H2O, Cells 500, 10% PMN, Protein 300, and Glucose 35. Which of the following is the best treatment?	Rifampicin, Isoniazid, Pyrazinamide, and Ethambutol
18	is an absolute contraindication for use of lumbar puncture	Raised ICP due to space-occupying lesion
19	A 13-year-old boy presenting with lower limb weakness for 3 days and then subsequently developed upper limb weakness and now he has become short of breath. Which of the following would not be found in this case?	brisk jaw reflex
20	Most common type of multiple sclerosis is	relapsing remitting MS
21	A patient has diplopia. On examination of horizontal gaze, you note failure of adduction of the right eye and nystagmus of the le abducting eye.	Right internucelar ophthalmoplagia

#	Fact cue	Answer
22	occurs in upper motor lesion	Babinski's sign
23	is used to predict the progression of multiple sclerosis	Incomplete recovery following relapse
24	Case of an old patient with very high-grade fever and headache, and after 2 days developed loss of consciousness, seizures, what's the most appropriate dx	Viral encephalitis
25	A girl with epilepsy presented to the clinical for seizure. She had night time trunk thrusting with high amplitude... During the morning, she was normal and had normal awake EEG	Pseudo seizure
26	is most specific and sensitive for TB Meningits	CSF PCR
27	A child presented to the neurological ward with left eye deviation, neck deviation to the same side, tonic shoulder extension on the same side with shoulder abduction. The person couldn't respond to any commands during the attack.	Complex focal seizure
28	A patient presents with sudden severe headache and neck stiffness, but no fever.	Nimodipine
29	tests best confirms a diagnosis of Myasthenia Gravis	single fibre electromyography
30	patient presents with right-sided weakness and atrial fibrillation. What to give	Warfarin
31	A known case of migraine develops irritability, confusion, food cravings, fatigue, problems concentration. Which phase of migraine is this	Prodromal phase
32	The ability to walk along a straight line is most often impaired with	Cerebellar dysfunction
33	don't commonly occur in Multiple Sclerosis	seizure
34	The upper motor neuron impairment produces the following change of muscles tone	Spasticity
35	Case of a patient with fever, neck stiffness and Kernig's sign positive What's the most appropriate dx	Bacterial Meningitis
36	medication does not cross the blood-brain barrier	Tetracycline
37	A patient with progressive temple pain with tenderness on the temples. He recently (yesterday) developed decreased visual acuity.	Giant cell arteritis
38	A young female presented with abnormal shaking movement of the body with up rolling of the eyes and urinated over herself. After that she had confusion and a headache. Which of the following is WRONG regarding her management?	Start loading dose of anti-epileptic
39	A 40-year-old female came to the consultation department complaining of left upper facial weakness. She said that 6 months ago, she had a similar complaint of right facial weakness. What is the most likely explanation for her symptoms?	multiple sclerosis
40	A 20-year-old woman presents the clinic with fever and back pain. Shegradually developed lower limb weakness and urinary incontinence. What isthe cause of this?	Dorsal Spine
41	MS medications is safe for a pregnant lady	glatiramir acetate
42	is most likely to be associated with Multiple Sclerosis	internuclear ophthalmoplegia
43	A patient with very bad preorbital headache with watery eyes, lacrimation. The headache was recurrent and would last for 30 mins. He had a similar previous headache last year that resolved without treatment. What is the most likely diagnosis?	Cluster headache
44	In a patient with trigeminal autonomic cephalgias, the pathway that causes lacrimation, nasal congestion, rhinorrhoea	Parasympathetic of superior salivatory nucleus
45	A 60 years patient with poorly controlled diabetes develops inability to move eye upwards and downwards and drooping of the eyelid on one side	Oculomotor palsy
46	A patient with optic neuritis and weakness (MS case). The visual evoked potential would show?	Prolonged P100

#	Fact cue	Answer
47	A long case. is true regarding polymyositis	Associated with proximal muscle weakness
48	Case of a patient with diabetes and sudden onset facial paralysis with weakness, what's the most appropriate dx?	Ischemic stroke
49	categories of medication is typically used to reduce the frequency and severity of episodes of hypokalemic periodic paralysis	Carbonic anhydrase inhibitors
50	The cauda equina is located	at the end of the spinal cord below the first lumbar vertebra
51	A 70 year old woman is brought in for seizure. Her son says she's been unusually forgetful and it has culminated in this episode of confusion. She has a fever of 38.2C. She develops seizures, hyperreflexia and upwards plantar response. What's the most likely cause?	Bacterial meningitis
52	A patient complains of le eye pain and visual defects. On examination, you note a defect in the central visual field of the le ft eye	Left optic neuritis
53	By presentations can you differentiate between spinal injury and peripheral polyneuropathy	sphincter problems
54	A case presents with diplopia and proximal muscle weakness that is worsened with exertion and better with rest. On examination you note ptosis and weakness but normal reflexes. Which of the following is the most appropriate initial investigation?	Antibodies to acetylcholine esterase
55	The presence of ataxia suggests damage to any of the following EXCEPT	Vagal nerve
56	A patient develops a defect in the nasal half of the right eye and temporal half of the left eye. What's the lesion?	Right occipital infarction
57	A child presented with the teacher saying he would present with frequent blank stares for a few seconds and would return to normal. On EEG, there was 3Hz spikes and waves.	Absence seizure
58	In a patient with status epilepticus, is not done acutely	haloperidol 10mg IM
59	In disorders would an elevation of the trapezius muscle due to poor fixation of the shoulder apparatus most commonly be seen on physical examination	Facioscapulohumeral muscular dystrophy
60	A 27-year-old man presented with 6 weeks of weakness and diplopia. He said he has trouble climbing stairs, especially at the end of the day. Which of the following tests is not appropriate test for diagnosis?	LP
61	A 46-year-old woman presented with a 9-month history of gait imbalance and pain and tingling in her legs. She had gastric bypass surgery 10 years prior. On examination, she had a wide-based gait and profoundly impaired joint position sense. Her family also reported she had become irritable recently. Which of the following investigations is most likely confirming the cause of her problems?	Blood test for vit b 12
62	Generalized tonic clonic seizure lasts less than 2 minutes True or false	T
63	is an abnormal cerebrospinal fluid (CSF) finding	Glucose level that is less than 35% of the plasma glucose level
64	Hemorrhagic stroke is more common than ischemic stroke True or false	F
65	All the following are true about evoked potential (EP) investigations in neurology except	The main use of auditory EP is in assessing the integrity of the cerebral cortex.

#	Fact cue	Answer
66	A 35-year-old female presented with poor coordination affecting her right arm, developing over the past 6-8 days. About 5 years before, she had sudden loss of vision in her right eye. Neurological examination showed some reduction of visual acuity in her right eye, jerk nystagmus of the abducting eyes and brisk jaw jerk, hyperreflexia (arms and legs), and extensor plantar response bilaterally. She had past pointing with her right arm. What of the following investigations is not appropriate for diagnosis?	Blood test for vit b. 12
67	tests best confirms a diagnosis of myasthenia gravis	Single fiber electromyography
68	An 18-year-old man presented with severe headache, nuchal rigidity, and a fever. The patient was lethargic on examination with papilledema. He was started on broad spectrum antibiotics. Which of the following is the most appropriate first step in his management?	head CT
69	bad prognostic features of the R.A include all the following, except	rapid onset
70	Petit mal epilepsy typically last less than 10 seconds? True or false	T
71	The internal capsule is surrounded by the following except	Red nucleus

Neurosurgery (38 questions) (38 questions)

#	Fact cue	Answer
1	antenatal ultrasound showed cyst on the back of fetus	Meningeocele
2	A 43 year old man presents with a history of recurrent back pain . He was neurologically intact. His imaging showed spondylolisthesis of the lumbar spine 37%. What's the treatment?	Conservative management (bed rest, analgesics)
3	You have a 57-year-old carpenter. He presented to the outpatient clinic due to severe neck pain. He reported that, at the beginning, the pain was mild but now it's much worse. T...	ACDf
4	A patient with pinpoint pupil, not reacting to light stimuli. He had impaired doll phenomena. It indicates damage to which of the following?	Pons
5	What is the cardinal feature of raised intracranial pressure	Papilledema
6	Stenosis of sylvian duct of brain	non-communicating hydrocephalus
7	Most common brain tumour in adults	Glioma
8	In spinal shock should be checked	bulbocavernous reflex
9	A 30 year old female patient develops back pain a fter exercising a fter 2 days.	Management with bed rest, muscle relaxant and analgesia.
10	A young-age male, a victim of RTA, suffering from paraplegia and internal abdominal bleeding. The best option to prevent thrombosis for him is:	Compression devices
11	A mother came to an antenatal checkup, in the US, a sac was discovered on the spine of the fetus. The general condition of the fetus and the mother is good. is true	Genetic cause is the most common cause for it
12	True regarding cavitron ultrasonic aspirator (cus	? a. The tip of ultrasound should be in direct contact with the tissue
13	tends to seed to the spinal cord the most	Meduloblastoma
14	The swelling in a patient with Hydrocephalus is due to	Periventricular Edema

#	Fact cue	Answer
15	True regarding encephalocele	It composed of dura without bony defect
16	A young male came to the ER, after a knife stabbing to the posterior neck, complaining from the inability to move his right side of the body and on examination there was loss of sensation on the left side. The type of lesion is	Brown-Sequard syndrom
17	A case of an infant presented with unilateral deformity of head. Which of the following is true?	Surgery is the management process f choice.
18	A patient a er an RTA had 80/60 mmHg blood pressure, and lower limb weakness. What's the diagnosis?	Neurogenic shock
19	The only tumour that can be caused by traum	Meningioma
20	Self-reporting of scale of pain consists of how many vignettes	11
21	True regarding gamma knife	Doesn't provide histological diagnosis
22	A patient who could localise to pain, open his eyes to command and had irrelevant speech. What is his Glasgow Coma Scale?	11
23	isn't an indication for Brain CT	Age more than 40 years
24	Sensory loss in the anterior lower thighs, knees and medial leg. nerve root is most likely affected	L4
25	A patient sustains a skull fracture. Which of the following is true regarding fractures?	A depressed fracture is said to occur when the outer plate of the fragment lies on level with the inner plate
26	A 12 year old sustained a direct head injury. He was initially conscious but lost consciousness a fter 2 hours. Which of the following hematomas/hemorrhages would most likely be responsible for this case?	Extradural hematoma
27	A patient presented to the emergency department a fter a head injury and was unconscious. Upon arrival, he regained consciousness and explained to the doctor that... A fter 10 minutes, he lost consciousness again.	Lucid interval
28	Most common location for brain tumour in children	Infratentorial region
29	which of these doesn't occur in an acceleration/deceleration head injury	epidural hematoma
30	A patient came to neurological consultation with repetitive lower back pain after lifting heavy object. His knee reflex (+) what is next step in management?	MRI of lumbar spine
31	A patient presented the emergency department a fter a head injury. He had briefly lost consciousness and regained it and presented with a GCS = 15.	Concussion
32	A neonate presents with hydrocephalus. Which of the following signs or symptoms would not be seen in this neonate?	signs of increased ICP
33	brain tumors has the highest risk of recurrence a er surgery	GBM
34	The evaluation of a patient with head injury is mainly by	all of the above (Options: A. GCS; B. Age; C. CT scan; D. all of the above)
35	A dentist showed you his MRI. Which nerve root is involved	C4
36	A patient presented with lower limb weakness, later on develops urinary incontinence and loss of perianal sensation. There is loss of sensation over dorsum of the foot and weakness of plantar flexion.	Cauda equina syndrome
37	What's the best Motor response of a patient who could localize the pain	5
38	A two months child presented with bilateral feet deformities, criteria is not related to rocker bottom deformity	The sole is flat

Psychiatry (58 questions) (58 questions)

#	Fact cue	Answer
1	A case that said i want to do a job that away from others, even for lower payments, what's the dx?	Avoidant PD
2	Which statement is false regarding the treatment of anxiety	Buspirone is a partial agonist acting on 5-HTA2 receptors
3	Which one is wrong regarding anxiety disorders	They are a group of mental disorders characterized by the presence of depression as the main prominent symptom
4	Which one is wrong regarding the effect of substances on sexual desire	Narcotics increase sexual desire
5	case that concerned about having serious illness like stomach cancer however she was young and did not has any significant symptoms, what is diagnosis ?	Illness anxiety
6	Diagnosis of a male who blames people for mistakes, has aggressive activity and causes legal issues	Antisocial PD
7	A Case who insisted on gettng a certain drug therapy, however his clinical symptoms and investigations don't line up with his story or account of pain, what is the diagnosis? (Cross matching)	Malingering
8	What percentage of ASD have a good prognosis	10-20%
9	A case that has a mental illness and received medication for it, then developed chewing of mouth, what is diagnosis?	Tardiv dyskinesia
10	A woman who is shy, introverted, and very sensitive to criticism avoids situations in which criticism may be given. She has which type of personality disorder	Avoidant personality disorder
11	You are concerned about a 78 years old man as he did not attend his diabetic clinic appointment. You undertake a home visit. His MMSE score is 14/30 and he is oriented to place...	Minimize cardiovascular risk factor
12	Treatment of smoking cessation is	nicotine replacement therapy (NRT) and social support.
13	A patient who previously participated in a war, during the war he lost his friend and now he did not remember that he lost his friend during the war, what is diagnosis	Dissociative amnesia
14	A 43-year-old businessman presents to hospital after going for some drinks with colleagues from work. He is restless, stating that he doesn't need to be in hospital and asking when he can go home. On examination he is tachycardic at 120 beats per minute with dilated pupils. Whilst you are examining him he starts to have episodic chest pain	Cocaine
15	What do you advise a pregnant woman who is dependent on heroin	Methadone maintenance
16	Regarding obsessive-compulsive disorder, all are true except	Egodystonic in OCD means the thoughts are familiar to the patients thinking
17	an obvious case of GAD) ... is NOT consistentwith the diagnosis	Hallucinations
18	A case diagnosed with sever ADHD, what is the treatment?	behavioral therapy
19	A case that was spending alot of money and full energy, decrease sleeping time, what is the diagnosis?	Mania
20	A woman is jealous of her husband and has sued him for infidelity. She functions well at work but checks his phone over and over. What's the diagnosis	Jealous delusional disorder
21	A long case. is the first line management of generalized anxiety disorder	cognitive behavioral therapy
22	The daughter of a 72 year old man asks you to see him at home as he has been losing weight and no longer leaves the house. When you visit he appears dishevelled.You know him wel...	Sertraline
23	On your way home from work you greet a well-known elderly patient who doesn't recognise you. He says he was on the way to meet some friends but has become lost and says he's bee...	Donepezil

#	Fact cue	Answer
24	A patient said: I started withdrawing from family, no longer enjoying activities I used to enjoy, think slowly and talk in a slow and monotonous way. What's this condition called?	Negative symptoms
25	A patient is admitted to hospital, receive a medication then developed galactorrhea , so which medication can cause it?	neuroleptic
26	A child has social anxiety and misses school for 3 weeks, what should you do as management?	Assist the child in returning to school immediately with family support.
27	Regarding psychiatric emergencies, which one is incorrect	In schizophrenic patients experiencing a relapse, the parenteral route takes priority over the oral route.
28	A 62-year-old woman is brought to hospital having suffered a fall in the street. On arrival she is disorientated and complaining of chest pain. An ECG performed demonstrates a prolonged QT interval and U waves. On examination she exhibits a lateral gaze palsy and is very unsteady on her feet.	Alcohol
29	A patient is an attention seeker, acts sexually seductive for approval and attention, exaggerates relationships and is very suggestible. Which personality disorder is most fitting?	Histrionic personality disorder
30	Lifetime prevalence of schizophrenia is	1%
31	order from best to worst prognosis	schizoaffective, schizophreniform, schizophrenia
32	A case with resistance schizophrenia, which medication used?	clozapine
33	A Case who had a variety of symptoms to which he gets admieed for, however staff discovered he was playing with the vital monitors, no signs of external gain. What is the diagnosis? (Cross matching)	-Facttious disorder
34	A Case who kept coming to the hospital thinking that he has cancer or other disease, and insists on being tested despite all his findings and investigations turning out to be negative. What is the diagnosis?? (cross matching)	Illness anxiety disorder
35	Diagnosis of a woman who develops sudden severe anxiety when going to a crowded public mall, to the point where she stops leaving her home due to no escape	Agoraphobia
36	A 36-year-old cocaine user notices that the longer he uses the drug, the more of it he requires to achieve the same effect what's this called	Tolerance
37	A 52-year-old man is admitted to hospital as he was found wandering the streets confused and ‘talking to himself'. He is well known to the local hospitals attending frequently with malnutrition and chest infections. On examination he has heavily injected conjunctiva, appears to be hearing voices and does not readily make eye contact. He is somnolent and exhibits a lack of interest in food or drink when brought to him	Marijuana
38	A case with features of major depressive disorder for two week then she got hallucinations, what is diagnosis?	major depressive disorder with psychosis
39	A patient on antipsychotics develops painful disabling turning of her neck to one side. Which of the following medications would help?	Diphenhydramine
40	Diagnosis of a female with feelings of emptiness and failure of interpersonal relationships	Borderline Personality Disorder
41	A patient reports episodes of sudden loss of muscle tone. Two recent examples were when he got news that his mother was diagnosed with cancer, and when he was warming up before a big race. What is his condition?	Narcolepsy
42	In social phobia the problem is	embarrassment
43	Main defect in mania is	eliated mood
44	A case of PTSD, which ones are his negative cognitions?	feelings of guilt and estrangement

#	Fact cue	Answer
45	drugs causes euphoria, arrythmia, hallucinations, and diaphoresis	Cocaine
46	A war veteran reports being unable to recall the events of a fight in which his best friend died during the war	Dissociative amnesia
47	A patient says: I started hearing voices saying "You're not good enough. You'll never be enough". What's this condition called?	2nd person auditory hallucination
48	patient presented to the psychiatric clinic with sadness, hopelessness, carelessness, inability to sleep, and etc... What is the most likely diagnosis?	Major depression disorder
49	drugs causes euphoria, arrythmia, hallucinations, and diaphoresis	Cocaine
50	is not a DSM criteria for bulimia nervosa	One attack per week for 3 months
51	is NOT a risk factor for obstructive sleep apnea syndrome	Female gender
52	Which drug is used to reverse the sedative effects of benzodiazepines	flumazenil
53	Patient into religious cults:	Prodromal phase of schizophrenia
54	A patient said: I started feeling as if people were after me, and the government was watching me. The voices persisted and told "they're coming for you". What's this condition called?	Persecutory delusion
55	What supports the diagnosis of pseudo dementia rather than dementia	Sudden onset of symptoms with rapid progression
56	patient present with sudden attacks of sweating, palpitations, and a feeling that he would die. Which of the following is the first line treatment for his condition?	CBT
57	A 24 year old female presents with sudden numbness and paralysis of her entire left arm. What is her condition?	Conversion disorder
58	25 year old male presented to urological out patient with sudden onset erectile dysfunction in past history he have depression treated by antidepressant medicine the second step is	the most common cause in this age is psychogenic so only reassurance

Pediatrics (282 questions) - Factual Memorization Verified

Built from the original source. Fact cue + answer for all 282 questions in Pediatrics (282 questions).

CNS (20 questions) (20 questions)

#	Fact cue	Answer
1	A patient with meningitis, what is next step in management:	start Dexamethasone + Cefotaxim + vancomycin
2	Whats the most appropriate treatment of a case of SIADH in meningitis	Fluid restriction
3	A 6-year-old child presents with drooling at night during sleep, without any other significant daytime symptoms. The child is otherwise healthy. What is the most likely diagnosis?	Benign Rolandic Epilepsy
4	the qeustion said that a child in crwoded area become fatigue and loss of consciousness	vasovagal attack
5	A child presented with staring of the eyes that lasted for 2-10 second that was associated with rapid recovery. An EEG showed a 3-hz spike and wave.	Generalized Seizure > Non-motor > Typical absence
6	A child presents with facial weakness and lower limb paralysis..which of the following is true about the disease?	isolation of campylobacter jejuni from stool
7	All of the following are risk factors for recurrence of febrile seizures, EXCEPT	short period of fever before seizure
8	is not a side-effect of carbamazepine	Hypernatraemia
9	Case 9 years girls with headache and fever for 12 hours , then developed rash over the trunk , hypotensive (90/60), febrile (40c), tachycardic 140 ,best next investigation?	Lumber puncture
10	A 16-month-old infant with 2-3 time upper and lower limb jerky movement during sleep only. He had no symptoms while awake and on neurological examination, he was normal. What is...	Benign sleep myoclonus
11	A 3-year-old infant, who on crying had flexing of the trunk with abduction of both upper limbs that occurred in clusters.	Generalized Seizure > Motor > Epileptic Spasm
12	Gillian-barre syndrome can be diagnosed by	Nerve conduction study and EMG
13	which drug was used in patient with history of gallstone	topiramate
14	True regarding infantile spasms	it is associated with agenesis of corpus callosum
15	A 3 year old child who had a difficult delivery was noted to be more clumsy than her peers by the nursery staff. She also had difficulty drawing shapes. What is the cause?	Cerebral palsy
16	treatments is commonly used to improve mobility in children with cerebral palsy (CP)	Botulinum toxin (Botox)
17	The most common cause of acquired hearing loss	Late group b strep meningitis
18	In a patient with trigeminal autonomic cephalgias, the pathway that causes lacrimation, nasal congestion, rhinorrhoea is:-	Parasympathetic of superior salivatory nucleus
19	patient with altered sleeping. which medication's side effect could be the cause	phenobarbital
20	A patient with a Hemi-convulsive seizure that lasted for 5 minutes, then after 2 months another attack of convulsion occurred. He had delayed development on motor examination. Which of the following is true?	It needs genetic sodium channel study

CVS (23 questions) (23 questions)

#	Fact cue	Answer
1	A 12-years old boy had syncope and breathlessness after exercise. On echocardiography he has hypertrophic chambers. His older brother died suddenly at age 30 years old	ICD and propanolol
2	A 6-month old infant with poor growth and difficulty feeding and a pan systolic murmur at left sternal border. He had a palpable liver 4 cm below the costal margin	Furosemide and Enalapril
3	is correct about a small VSD	The ECG is usually normal
4	Most likely diagnosis in a child presenting with fever, conjunctivitis, and red hands and feet	Kawasaki disease
5	A 4 months old infant was admitted to the emergency department for history of respiratory distress with poor oral intake and failure to thrive. On examination a pan systolic mur...	Large VSD
6	A case of inferior STEMI on ecg, then deteriorated suddenly, Whats the best next initial step	Transthoracic echocardiography
7	6 month old infant with poor feeding and failure to thrive, has SOB, Cough and diaphoresis, there is high pitched pan-systolic murmur 3/6 at LLSB and soT S2 Whats most likely di...	Large VSD
8	Shortly after a C-section delivery the child developed cyanosis and respiratory distress. After being given 100% oxygen, the patient had non improvement. On chest xray, there wa...	TGA
9	Most likely diagnosis in a child presenting with fever, conjunctivitis, and red hands and feet	Kawasaki disease
10	A 14 years old presents with cardiac murmur. He has a family history of sudden death. Echo revealed asymmetric septal hypertrophic cardiomyopathy of the upper part of the septum...	follow up in 6-12 months
11	A saturation of O2 deepens on patent alveolar wall, heightened membrane and airway. For a case if O2 was given, in conditions SpO2 will not improve	Cyanotic Congenital heart disease
12	A 12 year old presents asymptomatically to the pediatric clinic for routine examination with her mother. On auscultation, there's a second heart sound that was widely split and fixed, with an ejection systolic murmur in the left uppper eternal are	The chest X-ray showed increased pulmonary markings and normal cardiac thoracic ratio. An ECG showed an incomplete RBB
13	5 years old child diagnosed with VSD has sweating and poor growth, next step	surgical consultation for defect correction.
14	A case presents with cyanosis, and echocardiography demonstrates ventricular septal defect, pulmonary septal defect, overriding aorta and right ventricular hypertrophy. What's the correct diagnosis?	Tetralogy of Fallot
15	An infant presented with ejection systolic murmur on left sternal border, he was diagnosed as having Atrial septal defect, ECG was requested, which of the following is not a finding:	Upright T wave in V1
16	Which of the followings is not a feature of innocent murmur	Diastolic
17	A case of cyanosis and typical features of patent ductus arteriosus (PDA), treatment?	Ibuprofen
18	A child is brought in for evaluation of a heart murmur. The murmur is soft, short, systolic, heard best at the left sternal border, and varies with position. There are no signs of heart disease such as cyanosis, failure to thrive, or abnormal pulses. What is the most appropriate next step in management?	Reassure only
19	A case of TGA given, what's the management?	Arterial switch operation
20	A 3-week old male infant with TOF, what will be the management	BT shunt
21	patient presented to the outpatient department for a routine check-up. On precordium examination, a thrill was at the left sternal angle. On auscultation, a pansystolic murmur was heard all over the precordium. What is the most likely diagnosis?	VSD
22	A case of VSD with severe failure to thrive	PA band or VSD correction

#	Fact cue	Answer
23	A 2 days old baby presents with breathlessness and poor feeding. On examination, he has a soft murmur with a single and somewhat loud second heart sound, a 4-cm liver and her O2...	PGE2

GIT (27 questions) (27 questions)

#	Fact cue	Answer
1	A 9 month old child, what index is used commonly to assess nutritional state at this age	Weight for age
2	girl with symmetrical leg weakness, not in upper,,had diarrhea, is the most likely microorganism	Campylobacter
3	A 3 years old child who is breast fed presented with diarrhea and he is thriving well and has no illness. What's is the most likely explanation for his condition	Non-pathologic
4	is an indication to screen for cystic fibrosis in a newborn	Meconium Ileus
5	A patient with severe chest infection was admitted and treated with IV antibiotics for 5 days. He then developed watery diarrhoea. which microorganism is responsible	C difficile
6	A child gets watery diarrhoea in the winter months. Several of his schoolmates at the nursery have developed similar symptoms	Rotavirus
7	child with kerosine ingestion brought to hospital and he's feeling good what's the next step?	observer in hospital for 24 hour
8	ReSoMal is used for management of dehydration in severe acute malnutrition, compared to standard ORS, ReSoMal is characterized by	Lower sodium
9	a patient with sign and symptoms of refeeding syndrome , which one will be detected?	Hypokalemia
10	Child on breast feeding mother start weaning and gave him food after that he developed diarrhoea :	celiac disease
11	A child has a recurrent wheezy chest and chronic diarrhea. Which of the following tests are you going to use to confirm your diagnosis?	Sweat test
12	What is the most appropriate treatment for a child developing bloody diarrhea and cramping pain after taking amoxicillin for an ear infection	Oral vancomycin
13	A patient with severe chest infection was admitted and treated with IV antibiotics for 5 days. He then developed watery diarrhoea	Clostridium Difficile
14	is not a part of management in infantile colic	extensive investigation
15	is most indicative of severe bowel ischemia in a patient	Septic shock
16	is correct regarding malnutrition	Delay weight for height indicates acute malnutrition
17	What is the most appropriate fluid management for a 10 kg child with diarrhea, no fever, and some dehydration	750 mL over 4 hours
18	A child is receiving pancreatic enzyme replacement therapy. Which of the following conditions is the most likely indication for this treatment?	Cystic Fibrosis
19	According to the IMCI guidelines for management of acute gastroenteritis, what are the clinical features that should be depended on for classifying severity of dehydration	Sunken eyes, skin turgor, thirst, general condition
20	A child with bloody Diarrhoea, Anaemia, thrombocytopenia and elevated creatinine (NR not given).	E coli

#	Fact cue	Answer
21	This is a disease that affects the liver, pancreas, kidneys, recurrent chest infections and more. It is a cause of severe lung and GI conditions. fits	cystic fibrosis
22	A 4-week-old infant presents with non-bilious projectile vomiting and a palpable "olive-like" mass in the epigastrium. Laboratory tests reveal dehydration along with metabolic a...	Correction of dehydration and electrolyte imbalances (fluid replacement)
23	Diagnosis in 3 yr old child presented by his parents because of having loose stool which contain undigested food particles for 3 weeks, it's worse at the end of the day. no fami...	Chronic non specific diarrhea
24	findings is most indicative of malrotation in a child	Duodenojejunal (DJ) junction below the pylorus and to the right of the left vertebral pedicle
25	child gets watery diarrhoea in the winter months. Several of his schoolmates at the nursery have developed similar symptoms.	Rotavirus
26	A child with bloody Diarrhoea, Anaemia, thrombocytopenia, and elevated creatinine	e coli
27	A case of refeeding syndrome, what is the electrolyte abnormality?	Hypophosphatemia

GUT (10 questions) (10 questions)

#	Fact cue	Answer
1	a patient with repeated attacks of painless hematuria with each infection most common cause?:	IgA nephropathy
2	A child presented with peri-orbital edema and red urine. Took antibiotics for it. He later developed acute abdominal pain with tender and rigid abdomen. What's the most likely cause?	Peritonitis
3	In acute kidney injury, urinalysis can be sent as a basic investigation. In pre-renal azotemia, urinalysis is expected to show	High specific gravity
4	is an indication for biopsy in GN	Persistant hypocomplementemia
5	The finding which suggest renal damage in a patient with HUS is	Pyuria
6	a patient with HUS, what investigation will you send?	Peripheral blood smear and shistocytes
7	A young girl was brought by her mother to the outpatient clinic complaining of increased urinary frequency, which was noticed by her teacher at school. She also has burning pain during urination. What is the best initial investigation?	General urine examination (GUE)
8	is NOT an indication for dialysis in a patient developing acute kidney injury (AKI)	Metabolic alkalosis
9	What is your treatment for 1 year old child with nephrotic syndrome	Prednisolone 60 mg/m2 /day for 4 weeks
10	A child presented with severe abdominal pain and rash over the legs and buttocks. She had bleeding per rectum. Her CBC was normal – platelets were 270. Sonography normal. GUE-> 4 RBC. What is the most likely diagnosis:-	Henoch-schonlein purpura

General (growth, infections, genetic, endocrine...) (80 questions) (80 questions)

#	Fact cue	Answer
1	In patient with marfan syndrome, Cause of death	aortic dissection.
2	A child ingested their grandparents acetaminophen pills. Which of the following is the most common cause of mortality and morbidity in this case?	Hepatic injury

#	Fact cue	Answer
3	37-week neonate who was 4.5 kg and was born through C/S. Who had an open abdomen covered with a sac. His blood sugar level was 1.1mMol/L and had a temperature of 35.8 degrees. C...	Due to high insulin secretion
4	is the best description of varicella rash	Maculopapular rash then clear fluid containing vesicles. Rash keeps appearing and evolving
5	A child born at term, vaginal delivery and after 1 hour he has no health problems.	Vitamin K 1 mg IM
6	A child developed cough, fever, conjunctivitis, and a rash taht spread from head to the trunks. His pediatrician diagnosed him as Measles	Vitamin A 100 000 to 200 000 IU
7	A child developed cough, fever, conjunctivitis and a rash that spread from the head to the trunks. His pediatrician diagnosed him with Measles.?	Vitamin A 100 000 to 200 000 IU
8	A child with sickness and elevated glucose but normal HBA1C and ketones	No Insulin
9	A child with the ability to draw a correct circle and cross. He could go up the stairs on one foot at a time.	4 years
10	A neonate had a high fever and had seizure. After few days he developed rash over the body. What's the most likely diagnosis:-	Roseola infantum
11	A child was able to draw scribbles and a straight line, but not a circle.	2 years
12	An 11-year-old boy is the tallest child in his sixth-grade class. He is thin, has a high arched palate, mild scoliosis, and joint hyperextensibility. He is an honor student, and enjoys basketball but must use prescription sports goggles, so he can see the ball on the court. He is likely to have which of the following conditions?	Marfan syndrome
13	False regarding sexual abuse	Boys are more likely admit sexual abuse
14	Insulin used in an insulin pump	Rapid-acting insulin
15	Achondroplasia	AD
16	A 13-year-old female was brought to the pediatrics consultation ward by her parents who were worried that she was "falling behind" in her growth. She had not yet developed secondary sexual characteristics, she had broad chest, low posterior hair-line. What is the next step in management!	Chromosomal analysis
17	A 4 year old child with multiple skin vesicles maculas and papules had fever and this rash for a certain duration (chicken pox signs and symptoms), his 17 yr. old brother has just started to have fever and cough and he is not received vaccine, how do you manage the older brother ?	Acyclovir within the first 24 hours
18	A breast feeding mother got active TB. statements regarding this case is correct	The mother should temporarily stop breastfeeding for 2 weeks, take medications to treat TB during this time, and then resume breastfeeding.
19	A patient weighing 15 kg presents with signs and symptoms of diabetic ketoacidosis (DKA). What is the appropriate amount of fluid to give as a bolus in the first hour? (Recommended: 10–20 mL/kg of normal saline)	150 mL of normal saline
20	A 4 months baby with severe dehydration. After treatment in the first day he developed seizure. What is the most likely cause of the seizure:-	Na 156
21	20 month child, mile stone inability should be take in consideration	Walks unsteadily
22	A 1-year-old child presents with pallor. When assessing for potential causes of anemia, which of the following is the most important question to ask the mother?	What is the child's diet like?
23	oral and pharyngeal ulcer, what is the causative agent	coxakie virus.

#	Fact cue	Answer
24	Most likely diagnosis in a female patient with delayed growth, hyperconvex nails, cubitus valgus, and widely spaced nipples	Turner syndrome
25	Child on breast feeding mother infected with TB taking anti TB drugs	Breast feeding should be stopped for 2 weeks then continue
26	is not regarded as child abuse	Transportation
27	A 14-month-old boy brought to hospital by his parent they complained about their child started crawling and sitting without support with curved back last month, could be false a...	S. alkaline phosphate is normal.
28	A known case of type 1 diabetes mellitus received insulin and had breakfast as usual. After going to school and playing with other children, he suddenly developed sweating, tach...	Random blood sugar (RBS)
29	A 3-year-old Asian boy has a 4-week history of generalized malaise and fever. One week before his current presentation, his mother noticed a rash over his left cheek and a lump in front of his left ear. Initial examination revealed a macular popular rash over the left cheek and a non-tender lump in front of the left ear. Investigations reveal Hb 9g%, WCC 15 x 10 /l (neutrophils 72%, lymphocytes 20%, monocytes 8%, blood film ‘a few atypical monocytes') and CRP 40 mg/l. What is the most likely cause?	glandular fever
30	A patient presented with painful peri-oral blisters that had begun to crust. What is the most likely cause?	HSV
31	A child that can scribble and build a tower of blocks.	18 months
32	A 4 months old baby, active and happy turns over from supine to prone, coos. He's been brought in by his parents reporting he mostly uses his left hand and mouth with that hand...	Concern for handedness at this age
33	A 15-year-old girl with diabetes is experiencing shaking (tremor), sweating, and drowsiness. She's been busy with exams and skipped her breakfast.	Hypoglycemia in type 1 diabetes mellitus
34	A 14-year-old boy was at the 99th centile for his height. He had lens dislocation and, when asked to, he could bring his arms behind his back and make his elbows touch Which of the following is the most common cause of mortality in this patient?	Dilatation of Aorta
35	A child with red eyes that has had fever and rash that started at the back of the head and spread to the trunk.	Measles
36	At what age does the pubertal growth spurt typically occur in girls	12 years
37	9 months old child, was brought due to poor growth who's on the 5th cen)le?... and 10th cen)le...? Whats the most important factor of growth in this case	Nutrition
38	A 13-year-old female was brought to the pediatrics consultation ward by her parents who were worried that she was "falling behind" in her growth. She had not yet developed secondary sexual characteristics, she had broad chest, low posterior hair-line. What is the next step in management?	Look for Chromosomal abnormalities
39	An exclusively breastfed baby is brought by his mother with concerns that he is not growing as well as his peers. What is the first step in management	Plot the measurements on a growth chart
40	Prophylactic antibiotics are given at which grade of VUR	4
41	A young girl was brought by her mother complaining of weight loss, increased urination, and excessive thirst. What is the best initial investigation?	Random blood sugar (RBS) and HbA1c

#	Fact cue	Answer
42	A child was brought to the pediatrics clinic by their parents. The child was complaining from symmetrical short stature. 5-months old and was at <5% for the Height, Weight, and Head Circumference. At birth, the child was LBW (1000g). The child was a planned pregnancy with the mother have good prenatal care. On examination and history, the patient had normal finding. On the 30th week, the mother got into a road traffic accident and entered premature labour. What is the most likely of his low weight?	Normal growth of prematurity
43	A known case of diabetes mellitus presented with rapid breathing, abdominal pain, vomiting, and decreased level of consciousness. What is the investigation of choice	Arterial blood gases (ABG)
44	Which is MOST commonly used for growth assessment	Weight for age
45	A case of type 1 diabetes mellitus (DM), parents ask about the cause, how do you answer them?	autoimmune disease causing destruction of pancreatic Beta cells, caused be genetic and environmental factors
46	Insulin used in an insulin pump	Rapid-acting insulin
47	A 3-year-old child was brought to the pediatrician by their stepmother for a complaint of leg pain. An x-ray showed a spiral fracture of the femur. There was also a previous femur fracture in the other leg. What is the cause?	Child abuse
48	3-year-old child with a history of Acetaminophen ingestion. Which of the following is the most likely cause of mortality and morbidity in this case?	hepatic injury
49	Child on breast feeding mother infected with TB taking anti TB drugs what should the mother do ?	breast feeding should be stopped for 2 weeks then continue
50	An obese patient presents with constipation, low appetite, and cold intolerance. What is the most likely diagnosis?	Hypothyroidism
51	A 9-year-old child diagnosed with generalized tonic-clonic epilepsy was taking phenobarbital as treatment. He had had leg pain and bowing of leg. He was known for poor and irregular check-up. What would you consider in the next visit?	Serum Vitamin D
52	A 3-year-old child has a persistent fever, irritability, and a macular rash. There is also bilateral non-purulent conjunctivitis, unilateral lymphadenopathy, and a red tongue. What is the most likely diagnosis?	kawasaki disease
53	A child with bowing of legs and seizure. On the lab test he had a calcium level of 5.5 mg (Normal range was given and was low).	Oral vitamin D and IV calcium
54	Growth hormone has a short serum half-life, so is measured as an analogue for it	Insulin-like Growth Factor
55	Which one is best after paracetamol overdose after 6 hours	Whole bowel irrigation
56	rash descriptions is false	measles; petechial rash
57	Which isnt important in the vitamin d history	Bowel motion
58	breast feeding mother with mastitis and child with which oral white patches	Candidiasis
59	A child with hypotonia and delayed development, macroglossia and simian crease. What is the best confirmatory Test?	Karyotype analysis
60	is true for X-linked recessive disorders	Girls are usually carriers
61	A child with a pruritic maculopapular rash. It later progressed to a vesicle that contained fluid.	Chickenpox
62	A 6 week old infant with poor growth, jaundice, hepatosplenomegaly, hypotonia and lethargy, and bilateral cataracts	galactosemia
63	One of the principal pathologic features in fatal anaphylaxis is	Acute bronchial obstruction
64	child saying dada with no meaning	9 months

#	Fact cue	Answer
65	A young child who is a known case of type 1 diabetes on treatment, develops nausea and vomiting and abdominal pain. His RBS is 350 mg/dL, and his urine ketones were 4+. He was admitted to the hospital, where he received treatment with IV fluid and insulin, after 10 hours of treatment he developed confusion and drowsiness, and on examination his BP is elevated and his pulse rate is 50 bpm.	Diabetic ketoacidosis complicated by cerebral edema
66	A 3 days baby with ambiguous genital. He is most likely to have:-	High K low Na
67	An 11 year old child was brought by his parents because they were concerned as he was shorter than his classmates. He was their only son and they were worried about him. His height was just below the 5th centile. On examination, you find that his scrotum is not developed fully and he has scanty pubic hair. Radiologically, his bone age is 2 years behind. His mid-parenteral height is 168 cm. What is the most likely diagnosis?	Constitutional short stature
68	Route of transmission of polio virus is	feco-oral
69	An infant was diagnosed with Down syndrome due to Translocation, which of the following is correct:	Parental chromosomal analysis is recommended
70	A child presents with a history of prodromal features (cant remember them), and followed by swelling over the jaw and neck. There is tenderness over the jaw. The child hasn't had full vaccinations. What's the diagnosis and does the child need to be isolated?	Mumps parotitis/Yes
71	A child develops fever, cough, and SOB. On examination, he has bronchial breathing and dullness on percussion in the right lower lung field. His RBS is 350 mg/dL. His HbA1c is normal.	Stress hyperglycemia
72	is the most common form of child mistreatment	Neglect
73	An infant who could move his head to follow an object. He could not sit on his own. He smiled when prodded. His head was raised 45 degrees. He could coo. What is the child's developmental age?	3 month
74	A newly diagnosed DM type 1	Needs multiple insulin injections, including multiple short-acting and one long-acting insulin
75	A newborn has bilateral cataracts and microphthalmia, intrauterine growth retardation, hemorrhagic skin lesions scattered throughout the body, and a harsh systolic murmur heard...	Rubella
76	Which type of shock is associated with low blood levels	Hypovolemic shock
77	A 3-year-old girl presented for the first time with type I diabetes mellitus in diabetic ketoacidosis. She was dehydrated with rapid deep breathing. Intravenous fluid and insulin have been started and she started to be improved. 12 hours later, she vomited twice and became irritable with complaining of headache. O/E: The heart rate dropped from 120 regular beat/min to 77 regular beat/min. The following type of fluid should be used in management:	Mannitol 0.5-1 gram per kg over 10-15 minutes slow intravenous infusion.
78	A 29 months pregnant lady presented for regular check-up. She had a previous febrile illness with lacy rash over her body, joint pain, and red rash over the cheeks. the fetus is...	Generalized edema
79	A 2 year old child is brought into the ED with the complaint of one child was found with a bottle of (old furniture polish) a few hours earlier in the day. The most likely explanation for this child's condition is:	Hydrocarbon aspiration
80	A mother came to the growth clinic with her twin children (1 boy, 1 girl) worries that her daughter is 5cm longer than her brother and is worried that there is a growth defect....	12

Hematology (26 questions) (26 questions)

#	Fact cue	Answer
1	Cryoprecipitate contains all except	Factor V

#	Fact cue	Answer
2	A 3-year-old child with down syndrome presents with fever, easy fatiguability, pallor, and gum bleeding for ? days duration. On examination, you found hepatosplenomegaly. What is the most probable diagnosis?	Leukemia
3	is not a risk factor for iron deficiency anemia (IDA)	Isoniazid use
4	A case of hemophilia A on factor VIII replacement has poor control and continuous bleeding, what's the next step of management?	Factor VII replacement
5	A child with mild soft splenomegaly and cervical lymphadenopathy. They had fever and weight loss.	Lymphoma
6	A child with Fanconi anaemia. Which of the following is true regarding it?	Allogenic stem cell transplantation
7	Risk factor for iron deficiency anemia (ida) in children	Prematurity
8	A four year old boy presents to the GP surgery with his mother. He has been feeling unwell for the past 3 weeks, with fevers, headaches and tiredness. His mother is concerned th...	ALL (Options: A. ALL; B. EBV; C. ITP; D. Lymphoma; E. Aplastic anemia)
9	A boy with a family history of bleeding disorders requires preoperative evaluation. is used to screen for bleeding disorders in this patient	aPPT
10	Patient with hemoglobin 10mg/dl and microcytic. The serum iron normal, TIBC normal, serum feritin and electrophoresis are normal	Alpha thelasemia trait
11	A child with pallor and not active. With frequent coughing and infection. He had splenomegaly. He also had central abdominal mass. CBC was normal. What is the best next action	CBC, BMA and BM biopsy
12	5-year old girl was slapped on her cheeks by her brother and developed cyanosis on her cheeks. A day later she developed petechial spot on her leg. No lymph enlargement on hepatomegaly ...What is the next step.	CBC, blood film & INR
13	does NOT cause hypochromic microcytic anemia	fanconi anemia
14	Ten-year-old girl is brought to the office for well-child examination. During the past two months, the patient has had fatigue, occasional vague abdominal pain, and unintentiona...	bone marrow aspiration and biopsy.
15	Case of sickle cell anemia, HGB: 5 mg/dl, which infec)on is most likely?	Parvovirus B19
16	A one year old boy had a routine CBP screening with microcytic hypo-chromic cells, result of electrophoresis was Hb A1 92%, HbA2 5.5 % and HbF 2.5 %, what is your diagnosis	Beta thalassemia trait
17	Massive transfusions don't results in	Hypercalcemia
18	patient with heavy menstrual cycle	increase BT
19	An infant who is only on goat milk, which of the following could be elevated in lab findings?	MCV
20	is not true about physiological anemia	Iron deficiency contributes
21	A child presented with shortness of breath with a mediastinal mass. He has night sweats and weight loss.	Lymphoma
22	Sickle cell disease	AR
23	will not occur in a case of iron deficiency anaemia	Increase reticulocyte count
24	child with Down syndrome presented with gum bleeding for a day in duration, her condition was associated with organomegaly, fever, weight loss and multiple bruisings on her limb. What is the diagnosis?	Leukemia
25	child present with severe bleeding after circumcision	increase PTT, normal PT
26	is not a risk factor for iron deficiency anemia (IDA)	Isoniazid use

Neonatology (43 questions) (43 questions)

#	Fact cue	Answer
1	The initial redistribution of blood in Asphyxia is to which organs	Heart, brain, adrenal glands
2	Grunting is when air is exhaled against a partiAally closed glottjs, It is most commonly found in	Hyaline membrane disease
3	In regards to umbilical granuloma, which one is true:-	Represent incomplete epithelialization of tissue that persists after the cord separates
4	is NOT an indication to send for further investigations in a jaundice case	TSB more than 1 mg/d
5	A child able to draw scribbles and has started to build towers	2 years
6	A mother has a child with congenital CMV who's being followed up, and he's deaf and has intellectual deficit. She's concerned for her second pregnancy. What do you tell her	CMV antibodies pass to the fetus
7	Regarding sternocleidomastoid tumour, which one is false:-	Occur during caesarean section
8	A child able to draw scribbles and has started to build towers	18 months
9	A newborn develops jaundice within the first 18 hours of life. Laboratory tests reveal an elevated unconjugated bilirubin level. The mother's blood type is O+, and the baby's bl...	ABO Incompatibility
10	A 4-week-old infant presents with persistent jaundice, pale stools, and dark urine. Laboratory tests show direct (conjugated) hyperbilirubinemia. An abdominal ultrasound fails t...	Biliary Atresia
11	pregnant women with rupture of membrane at 12:00 am give birth to a child at 4:00am what bed side investigation is important to be done for her	O2 measurement
12	A 2 week old newborn has prolonged jaundice. He also has an umbilical hernia. What is the most likely diagnosis	Hypothyroidism
13	A patient with the ability to draw a correct circle and cross. He could go up the stair with one foot at a time	4 years
14	A newborn has hydrocephalus, chorioretinitis, intracranial calcifications, and anemia	Congenital toxoplasmosis
15	An infant with low-set ears, beaked nose, and oligohydramnios.	potter sequence
16	organs is spared in asymmetric IUGR	Brain
17	Not a complication of an infant of diabetic mother (iodm)	Hyperglycaemic convulsion
18	A pregnant mother has an older 2 year old son who has Cytomegalovirus. She is worried about her current pregnancy. What will you tell the mother	That the mother has developed antibodies that pass to the fetus
19	organs is spared in asymmetric IUGR	Brain
20	neonate, ill with jaundice and abnormal vital signs, presented with a swelling extending over the parietal bone and frontal bone. What's the correct diagnosis?	Subgaleal hematoma
21	You are called to the delivery of a 39-week infant, with no known maternal complications of pregnancy. Meconium was noted at the time of delivery. You arrive in the delivery roo...	Provide positive-pressure ventilation via bag-mask ventilation
22	A 3-year-old child was brought to the pediatrician by their stepmother for a complaint of leg pain. An x-ray showed a spiral fracture of the femur. There was also a previous femur fracture in the other leg. What is the cause?	Child Abuse
23	A neonate is well and healthym presents with abnormal body movements in the morning. Blood glucose is normal. Next investigation?	S. Calcium and magnesium

#	Fact cue	Answer
24	A newborn male is delivered via emergency C-section at 38 weeks due to fetal distress. At birth, the baby is limp, pale, and not crying. The following Apgar scores are recorded...	Apgar 3
25	A child was brought to the pediatrics clinic by their parents. The child was complaining from symmetrical short stature. 5-months old and was at <5% for the Height, Weight, and Head Circumference. At birth, the child was LBW (1000g). The child was a planned pregnancy with the mother have good prenatal care. On examination and history, the patient had normal finding. On the 30th week, the mother got into a road traffic accident and entered premature labour. What is the most likely of his low weight?	Normal growth of prematurity
26	What is the first concern for a premature infant weighing 800g	Respiratory distress syndrome
27	Not a complication of cold injury	Hyperglycaemia
28	Not a complication of cold injury	Hyperglycemia
29	Not a complication of an infant of diabetic mother (iodm)	Hyperglycemic convulsion
30	39 weeks neonate is just delivered by CS, (the remain of scenario goes with meconium aspiration), he has dyspnea and his heart rate was increased after vigorous stimulation, but...	Intubation and mechanical ventilation
31	A3-week- old neonate presented with history of prolonged jaundice and poor feeding. on examination: he has cataract, purpuric skin lesions, hepatosplenomegaly, and systolic murm...	IgM antibodies against rubella
32	15 year old child had 5 minutes of seizure. As she was awaiting her laboratory tests, she seized again. Which of the following can be used to stop her seizure activity?	diazepam
33	After delivery, you noticed the baby's arm adducted at the shoulder and internally rotated. What's the most likely site of injury:-	C5, C6, C7
34	is not a cause of jaundice in first 24 hr	Crigler najjar syndrome
35	A floppy child with flattened face and a single palmar crease	down syndrome
36	pregnant women with history of ruptured of membrane 24 hours is least likely for the child to get	Osteomyelitis
37	Most common congenital cause of duodenal obstruction	Type I atresia
38	A 30 weeks GA preterm baby develops respiratory distress, diagnosed as RDS and admitted to NICU. What's the next step in the NICU	O2 supplementation
39	does not affect the effectiveness of phototherapy	Skin color
40	An antenatal ultrasound showed pedal edema with a horse-shoe kidney	turner syndrome
41	You are called to the delivery of a 39-week infant, with no known maternal complications of pregnancy. Meconium was noted at the time of delivery. You arrive in the delivery roo...	Provide positive-pressure ventilation via bag-mask ventilation
42	A newborn has microcephaly, intracranial calcifications, hepatosplenomegaly, and a marked hyperbilirubinemia and thrombocytopenia	Congenital CMV
43	A 4 week old baby, thriving well on breast milk, has grown from 2.5 to 5 kg, has odd looking fancies and jaundice. What's the diagnosis	hypothyroidism

Pediatrics Surgery (21 questions) (21 questions)

#	Fact cue	Answer
1	Most common congenital cause of duodenal obstruction	Type I atresia
2	In regard to omphalocele, which one is true :-	Doesn't need urgent surgery
3	A child presented with a tender warm right submandibular mass. That wasabout 5cm. What's the most common microorganism cause?	Cat scratch disease
4	What is the most common presentation of Meckel's Diverticulum	Intestinal obstruction
5	tests is considered an essential feature of the preoperative evaluation and surgery decision of a patient with a suspected thyroglossal duct cyst	Cervical Ultrasound
6	Regarding intralobar pulmonary sequestration, is true	Blood supply derived from aorta
7	A preterm baby was being fed, then he suddenly deteriorates, develops bloody diarrhea, abdominal distension, and on imaging pneumatosis intestinalis was seen. What could be the...	Necrotizing enterocolitis
8	Where is the site of Omphalocele	Base of umbilicus
9	Regarding thyroglossal duct cyst, which one is true:-	Surgical excision is always required even if it was the only thyroid tissue
10	Intestinal atresia is secondary to late intrauterine conditions seen, except for	Appendicitis
11	is the most common site of tracheoesophageal fistula	Distal type
12	is the most common site for lobar emphysema	Left upper lobes
13	Gold standard for Hirschprung is biopsy showing absence of ganglion in myenteric and submucousal plexus. True or false	True
14	4 weeks old neonate presents with fever and respiratory distress. He's had coughing since birth worsened with food and develops cyanosis and has frothing	-tracheoesophageal fistula
15	A case of cholestasis and acholic stool,what's next?	Abdominal US
16	Regarding jejunoileal atresia, is not correct	Type 1 and 2 are associated with a V-shaped gap in the mesentery
17	A case with elevated pancreatic enzymes and dilated bile duct, what will be the management?	ERCP
18	4-day old presents with dark-coloured stool. most likely indicates severe Bowel Ischaemia	Septic Shock
19	All of the following are correct regarding anterior abdominal wall defect, EXCEPT	Gastroschisis has higher mortality
20	A case of choledochal cyst	MRCP
21	A child presented with recurrent RTI, X-ray showed elevation of diaphragm and liver pushed upwards to the chest. What is the best treatment?	plication of the elevated part

Respiratory (32 questions) (32 questions)

#	Fact cue	Answer
1	What is the most common cause of non-infectious Stridor in infancy	Laryngeomalacia
2	A 12-month-old child presented with a cough for five days with no danger signs, no chest indrawing, and no fever. Respiratory rate is 40 per minute. What is the classification	Pneumonia

#	Fact cue	Answer
3	All of the following is a management of Bronchiolitis, EXCEPT	Bronchodilator
4	A 2-year-old child presents with a barking cough, hoarseness of voice, and stridor. This is the second such episode, and there were no significant prodromal respiratory symptoms (e.g., fever, runny nose, or congestion) before onset. Which of the following is the most likely diagnosis?	Spasmodic Croup
5	Regarding simple pneumothorax, which one is false:-	Can expand
6	A child has cough and fever for a few days. Now he has severe SOB. WBC was 22,000 with mainly neutrophils. What is the likely diagnosis:-	Pneumonia
7	A 2-year-old presented at night with sudden wheezing that was localized to the right lung field and that was not preceded by fever or any respiratory illness. On chest X-ray there was right side hyper expansion.	Foreign body inhalation
8	The most common causative microorganism of croup is	Parainfluenza
9	child present with respiratory distress, toxic , leaning forward , drooling , with faint stridor ?	acute epiglottis
10	is the most common symptom of bronchiolitis	Fever
11	A child presents with fever, sore throat, and cervical lymphadenopathy. The child was prescribed antibiotics, after which they developed a widespread maculopapular rash. Which of the following is the most likely diagnosis?	Epstein-Barr Virus (EBV) Infection (Infectious Mononucleosis)
12	Case of otitis media, has penicillin allergy, which drug used next?	cephalosporin
13	child with epiglotitis what is the next step?	Intubation
14	A mother while preparing the food inside and her child is playing outside, heard the child chocking and coughing. Spo2 was 97%. What's the diagnosis:-	Foreign body inhalation
15	is a cause of strong cough in children	pertusis
16	14 year old child has a one week history of sore throat and dysphagia. Her condition has gradually gotten worse. She now has difficulty opening her mouth, worsened sore throat, and drooling. On examination, the uvula is pushed to the right.	peritonsillar abscess
17	What is the recommended treatment for mild to moderate bronchiolitis	Oxygen
18	When oxygen is given and PaO2 is not correct, then conditions growth this finding	Cyanotic CHD
19	A patient had coughing at night for the last 6 months with rash on his hand. His sister was diagnosed with atopic dermatitis last week and his brother had cow-milk allergy	atopic asthma
20	A child presented with earache , fever , irritability otoscopy showed red bulgy tympanic membrane diagnosed with acute oAAs media, and has allergy to penicillin, what is the Tx:	Cephalosporin
21	A child has a second attack of cough that occurs suddenly at night and was not preceded by upper respiratory tract infection. Which of the following is the diagnosis?	Spasmotic croup
22	Most common bacterial cause of severe sore throat and lymphadenopathy in children	Group A streptococcus
23	case term delivery , after delivery presented with dyspena and , then put on positive CPAP spo2 =98, after that developed cyanosis and decreased o2 saturation	Pneumothorax
24	Barking cough is not a feature in	Epiglottitis
25	A 6 years old fir presents with fever, chest pain, SOB, and sputum. Her examination revealed bronchial breathing and dullness in the right lower field. What's the most likely mi...	Streptococcus pneumoniae

#	Fact cue	Answer
26	What is the most common atypical cause of pneumonia in a child older than 5 years	Mycoplasma Pneumonia
27	child with a 3-day history of URTI and was admitted to hospital and developed bouts of coughing that would last for 15 seconds and ends with vomiting. The last time he had 2 bouts of cough that ended with cyanosis.	pertussis
28	In is the most common age group for foreign body inhalation	toddler
29	An unvaccinated 7-year-old with 3-days hx of URTI. Presented with high fever, inability to talk and inspiration Stridor and cyanosis. is the next step in management	Intubation
30	A child has a 20 day history of coughing. The cough comes in paroxysms, lasting long and ending in vomiting or sometimes cyanosis. The investigations are significant only for lymphocytosis.	Pertussis
31	Most common bacterial cause of severe sore throat and lymphadenopathy in children	Group A Streptococci
32	Which one is not true in acute suppurative otitis media	The bacteria commonly are pseudomonas aeruginosa, staph aureus

Surgery (350 questions) - Factual Memorization Verified

Built from the original source. Fact cue + answer for all 350 questions in Surgery (350 questions).

ENT (56 questions) (56 questions)

#	Fact cue	Answer
1	True regarding palatine tonsils	has 12-15 crypts
2	A young swimmer presents with earache, most likely diagnosis:	Diffuse otitis externa
3	About adenoid the pathology	the pathology is due to size and infection.
4	False regarding larynx anatomy	The framework of the larynx is made from bone
5	Choose the SINGLE most appropriate answer from the above list of options. (Cross match) Ear drop antibiotic	Otitis externa
6	Choose best option(cross matching) A 45 year old man presents with bilateral blockage, anosmia, nasal discharge, postnasal drip, and headache	Bilateral nasal polyp
7	Final 2023 Q53 - A 55-year-old patient presents with a persistent, watery discharge from their left nostril that started about a week after they hit their head during a fall. Th...	CSF Rhinorrhea
8	choose the SINGLE most appropriate answer from the above list of options.(Cross match) Indication for surgical intervention	Anterior and posterior nasal packing
9	Final 2023 Q 57 - A patient presents with a sudden onset of severe sore throat, fever, difficulty swallowing, and visibly red, swollen tonsils, possibly with white patches (exud...	Antibiotics play a key role in treatment primarily when a bacterial cause
10	Tonsillectomy indicated	5 real attacks in each year of the last 2 to 3 years
11	Child had profound sensorineural hearing loss, what is the best management	Cochlear implants
12	Rigid bronchoscopy is preferred over flexible bronchoscopy for the following	The removal of a bronchus intermedius foreign body from an infant.
13	Choose best option. (Cross matching) A25 year old lady with purulent discharge, facial pain and headache, nasal obstruction, and fever, following an episode of recent URTI	Acute rhinosinusitis
14	True about scarlet fever	It is a rare streptococcal infection with a specific toxin
15	Sneezing is most common feature	Allergic rhinitis
16	When does pain disappear in otitis media	Perforation of the drum
17	True regarding pure tone audiometry	Can be used to determine the site of pathology
18	Cause of acute diffuse otitis externa	Pseudomonas aeruginosa
19	A child presents with earache and fever following upper respiratory tract infection. What is the most likely diagnosis?	Acute suppurative otitis media
20	A child presents with a high fever, sore throat, a flushed face with paleness around the mouth, and a characteristic red, rough-textured rash described as feeling like sandpaper. The tongue may also appear bright red ("strawberry tongue").This clinical presentation, known as Scarlet Fever, is recognized primarily as:	A complication resulting from certain toxin-producing strains of Group A Streptococcus.
21	What is included in post-operative care of tracheostomy	Repeated cleaning and humidification
22	A young swimmer presents with earache, most likely diagnosis:	Diffuse otitis externa

#	Fact cue	Answer
23	False statement about regarding larynx physiology,	Straining is easy in people who have vocal cord paralysis
24	When is an emergency tracheostomy indicated	In upper airway obstruction
25	A 6-year-old child is evaluated for chronic, severe nasal obstruction. The child primarily breathes through the mouth, snores loudly at night, and sometimes pauses in breathing during sleep (apneas). Examination and imaging confirm markedly enlarged adenoids that significantly block the nasopharyngeal airway. In addition to alleviating nasal obstruction and sleep-disordered breathing, a primary indication for performing an adenoidectomy in such a child is to help prevent potential long-term:	Abnormalities in maxillofacial growth and dental occlusion
26	80 years old man developed hearing loss what is the most probable diagnosis	Presbycusis
27	A child develops headache, nasal obstruction, and other symptoms after swimming. Later, both his eyelids become swollen. What is the most likely diagnosis?	Ethmoid sinusitis
28	True regarding singer nodules	Consist of fibrous tissue covered by epithelium
29	is not a systemic cause for epistaxis	Sever septal deviation
30	is not a protective mechanism in the ear	Oval window
31	is the most informative investigation for diagnosing pediatric acute sinusitis	CT scan
32	A 60-year-old patient presents with a firm, tender mass in the upper neck. There is associated hoarseness, and the patient reports a persistent headache. What is the most likely cause?	Metastatic cancer to lymph nodes
33	Which one is correct about infectious mononucleosis	All of the followings are true (Options: A. Caused by EBV; B. Has long incubation period; C. Leads to severe sore throat for a duration of 2-4 weeks; D. Not responding to antibiotic is a feature; E. All of the followings are true)
34	A patient with unilateral polypoidal mass on lateral nasal wall, Dx	inverted papilloma
35	True regarding quinsy	All above (Options: A. It is a collection of pus between the fibrous capsule of the tonsil & the superior constrictor muscle; B. It is mainly a complication of acute tonsillitis; however, it sometimes presents without a previous history of tonsillitis.; C. It's usually seen in young adults & rarely in children.; D. Can cause severe trismus (lock jaw); E. All above)
36	Which one is correct about tonsillitis	Nowadays amoxiclav and amoxicillin are used as treatment
37	What complication of tracheostomy occurs due to apical pleural injury	Pneumothorax
38	A 45-year-old farmer presents with a primary complaint of hoarseness that has gradually worsened over the last 6 months. He has a significant smoking history (20 pack-years) but denies any recent fever, cough, or symptoms of an upper respiratory infection. His voice quality is consistently rough and low-pitched. Fiberoptic laryngoscopy reveals bilateral, symmetrical, diffuse thickening and mild erythema (redness) along the entire length of the true vocal cords. Vocal cord mobility is normal, and there are no distinct nodules, masses, ulcers, or white patches (leukoplakia) visible.	Chronic Laryngitis
39	The causative organism for acute localized Otitis externa is	Staph aureus
40	is not a symptom of ear disease	Presbyopia

#	Fact cue	Answer
41	Regarding atticoantral chronic suppurative otitis media, which one is true	Cause scanty foul-smelling discharge
42	Final 2023 Q 58 - An 12-month-old infant presents for evaluation. The parents report that since the early months of life, the infant has had noisy breathing, characterized by a...	Laryngomalacia
43	choose the SINGLE most appropriate answer from the above list of options.(Cross matching) Recurrent epistaxis	Anticoagulant use
44	choose the SINGLE most appropriate answer from the above list of options.(Cross match) Child with unilateral blockage and offensive discharge	Foreign body
45	Choose the SINGLE most appropriate answer from the list of options.(cross match) Cholesteatoma	Bone erosion
46	Final 2023 Q 59 -A 5-year-old child presents to the emergency room with a rapidly progressing illness over the past few hours. Key findings include a high fever (39.5 ■C), sever...	Acute Epiglottitis
47	A patient presents with hoarseness and headache. The headache is localized to the forehead and worsens with bending forward. What is the most likely cause?	Acute sinusitis
48	choose the SINGLE most appropriate answer from the above list of options.(cross match) Self limited	Epistaxis in children
49	True about epistaxis	Mostly idiopathic
50	An old diabetic presents with earache, most likely diagnosis	Malignant otitis externa
51	Choose the SINGLE most appropriate answer from the above list of options.(cross matching) dry perforated eardrum	Myringoplasty is the treatment of choice
52	True about chonal atresia	More common in female
53	Which is correct regarding tracheostomy	it can be used to prevent aspiration
54	nerves doesnt supply the external ear	Corda tympani
55	The most common cause of hearing loss in children in the winter is	Middle ear effusion
56	watery nasal discharge is a finding in	Option E (could not safely map from original source)

General Surgery (1 questions) (1 questions)

#	Fact cue	Answer
1	is right regarding DPL(Diagnostic peritoneallavage)	DPL is very sensitive

Ophthalmology (63 questions) (63 questions)

#	Fact cue	Answer
1	What is the earliest sign of open angle glaucoma	Paracentral scotoma
2	what is associated with keratoconus	Irregular astigmatism
3	is a sign of advanced hypertensive retinopathy	Optic disc swelling
4	A patient presents with blurry vision after receiving eye drops for mydriasis. Ophthalmoscopic examination reveals no red reflex and a whitish color in the eye. What is the most likely diagnosis?	Dense mature cataract
5	A patient presents with a painful tender localized upper eyelid swelling. What's the diagnosis?	Stye

#	Fact cue	Answer
6	Mechanism of action of acetazolamide in glaucoma management	Decreases aqueous humor production
7	Adult inclusion conjunctivitis is characterised by	Follicular reaction of conjunctiva
8	Final 2023 Q 60 - Which statement accurately describes an aspect of managing acute nasal bone fractures in adult patients	Reduction of a displaced nasal fracture, when indicated for cosmetic or functional reasons, can frequently be performed under local anesthesia in adults
9	Case of a child/person with myopic refractive error, what can be used to treat it?	Excimer laser surgery
10	Head turn is seen in refractive errors	Mixed astigmatism
11	In a person with chronic diabetes, blurring of vision will occur due to ischemia in: (REPEAT)	Maculopathy
12	Not important in differentiation between pre septal and orbital cellulitis	Red eyelid swelling
13	operations is associated with a shorter rehabilitation time	Phacoemulsification
14	Regarding thyroid eye disease	Usually bilateral and symmetrical
15	"Trantas dots" are characteristic of which disease	Vernal keratoconjuctivitis
16	Treatment of choice in herpes simplex keratitis	Acyclovir
17	Treatment of pre-septal cellulitis	Tobramycin ointment and oral co-amoxiclav
18	The first line treatment of angle closure glaucoma is	IV mannitol
19	is a predisposing factor for bacterial conjunctivitis	Hot dry weather
20	transmits the function of the ciliary muscles to the lens	Lens zonule
21	Treatment of adult inclusion conjunctivitis	Tetracycline
22	Mechanism of action of acetazolamide in glaucoma management	Decreases aqueous humor production
23	A patient with adult inclusion conjunctivitis.What is the appropriate treatment?	Tetracycline eye ointment
24	is the correct flow of aqueous fluid? (REPEAT)	Posterior chamber → anterior chamber → trabecular meshwork → canal of Schlemm
25	A patient presents with a red eye due to seasonal allergy. What is the appropriate treatment?	Olopatadine
26	Cause behind micro aneurysm in case of diabetes	Ischaemia
27	Angle closure glaucoma causes	Secondary anterior cataract
28	is correct regarding monocular diplopia	Diagnosed by slit lamp microscopy and retinoscopy
29	Pre-auricular lymphadenopathy occurs in	Adenoviral conjunctivitis
30	A patient presents with sudden onset loss of vision, a painful red eye, and is also experiencing nausea and vomiting. What is the most likely diagnosis?	Primary closed-angle glaucoma
31	Not important in differentiation between pre septal and orbital cellulitis	Red eyelid swelling
32	is a risk factor for primary angle-closure glaucoma	Hypermetropia
33	What is the differentiating point between open angle and angle closure glaucoma	Depth of the anterior chamber

#	Fact cue	Answer
34	An elderly female patient with a history of diabetes presents a fter getting very angry with sudden loss of vision. On fundoscopy, there are diffuse retinal hemorrhages; macular edema; exudates; and dilated tortuous veins.	Central retinal vein occlusion
35	is pathognomonic of central retinal artery occlusion	Cherry red spot
36	best differentiates trachoma from other diseases	Herbert's pits
37	is a risk factor for open-angle glaucoma	High Myopia
38	a child develops painless eyelid swelling after an insect bite	Angioedema
39	What is the likely cause of central retinal artery occlusion in a young person	Hemoglobinopathy
40	A case presented with accumulation of pus in the anterior chamber(hypopyon), what is the most likely diagnosis?	Iritis
41	What is the most common microorganism that causes cataract is? (the question did not specify whether it meant congenital or in general)	Rubella
42	is an indication for corneal transplantation	Sever keratoconus
43	What does target intraocular pressure mean	No further damage to optic nerve or increase in visual field loss occurs
44	is correct regarding binocular diplopia	Vascular causes have sudden onset
45	The immediate treatment of angle closure glaucoma is	IV mannitol
46	Papilledema is a finding in	Cavernous sinus thrombosis
47	Female patient presents with headache, temporal tenderness and sudden loss of vision	Giant cell arteritis
48	A patient presents with nausea, vomiting, eye pain, and a shallow anterior chamber. What is the most likely diagnosis?	Acute closure glaucoma
49	A patient with a history of diabetes presents with sudden loss of vision. On fundoscopy, there is no view.	Vitreous haemorrhage
50	A patient with diabetes presents with diffuse hemorrhage, macular edema, and dilated veins in the retina. What is the most likely diagnosis?	Central retinal vein occlusion
51	An elderly patient presents with sudden, unilateral, painless loss of vision. Fundoscopy reveals retinal hemorrhages. What is the most likely diagnosis?	Central retinal vein occlusion
52	is not used as a treatment for chemical conjunctivitis	Vitamin (a)
53	In long-standing advanced hypertensive retinopathy there will be	Optic nerve swelling
54	is not a feature in adult inclusion conjunctivitis	Keratic precipitate
55	Pre-auricular lymphadenopathy occurs in	Adenoviral conjunctivitis
56	An elderly patient presents with sudden loss of vision. On examination, the retina looks pale and edematous while the fovea looks red.	Central retinal artery occlusion
57	A diabetic patient presents with poor vision. On fundoscopy, there is an absence of the red reflex. What is the most likely diagnosis	Hyperdense cataract
58	A patient presents with painful sudden loss of vision, and on fundoscopy there is optic disc edema and hyperemia.	Optic neuritis
59	True regarding diabetic cn iii palsy (this wasn't the way the question was worded but it is what it meant)	Treatment is by control of underlying disease and follow up

#	Fact cue	Answer
60	A 6 year old child brought to the clinic because of convergent squint and le eye 6/60 and right eye 6/6 visual acuity. What's the appropriate treatment?	Cycloplegic refraction and orthoptic treatment
61	A patient presents with sudden loss of vision with a preceding history of floaters and photopsia	Retinal detachment
62	Treatment of choice in seasonal allergic conjunctivitis	Olopatadine
63	A patient presents with a red eye and is diagnosed with herpes simplex keratitis. What is the appropriate treatment?	Acyclovir

Orthopedics (131 questions) (131 questions)

#	Fact cue	Answer
1	True regarding chondroblastoma	Affects primarily the epiphysis, usually of the proximal humerus, femur or tibia
2	is true about Giant cell bone tumor	Affect mature bones, rarely seen before the physis closure
3	A 6 year old child presents after a trauma, and is diagnosed with an undisplaced fracture of the radius and ulna. How to manage?	Splint above elbow
4	Best immobilization for a third metacarpal fracture	Safe immobilization
5	Not seen in developmental dysplasia of hip	Avascular necrosis of femoral head
6	Which if the following is not a feature of rock foot	Flat foot
7	is the most sensitive sign for DDH in a 6 months old child	Limited abduction in flexion
8	False regarding genu varum	Assessed by intermalleolar distance
9	Incorrect regarding nerve injuries	Crush injury results in axonotmesis
10	A child with red swallen metphysis and high WBC?	acute hematogenous osteomyelitis
11	A 3-year-old child presents with crying, painful extended and pronated elbow that they are unable to flex. What is the diagnosis?	Pulled elbow
12	A patient presents to the ER with a Monteggia Fracture-Dislocation. Which of the following is the best treatment?	restore the length of ulna surgically
13	Incorrect regarding nerve injuries	Crush injury results in axonotmesis
14	True about osteoid osteoma	It has no risk of malignant transformation.
15	Immobilization of fractures of long bones should include	Both proximal and distal joints
16	False regarding genu varum	Assessed by intermalleolar distance
17	A young patient with painful shoulder joint and Hx of overuse	acute tendinitis
18	A 65-year-old with femoral neck fracture of 6 weeks ago. What is the best treatment now?	Hemiarthroplasty
19	is true about osteochondroma	It stops growing with the end of the growth of the affected bone
20	is true of rocker bottom foot	Talus is vertical

#	Fact cue	Answer
21	A 50-year-old patient presents with an inability to lift the shoulder. The physical examination reveals a positive abduction paradox test. What is the most likely diagnosis?	Complete rotator cuff tear
22	Nerve injured in anterior shoulder dislocation	axillary nerve
23	The reconstruction in lateral collateral ligament tear means	Putting the tendon in place of ligaments
24	A 20 year old sustained an injury a while ago and got a fracture. He was treated by an osteopath. He now has cubitus valgus and has pain and tingling over the medial one and a half fingers. How do you treat it?	Anterior transfer of ulnar nerve
25	a patient with undisplayed distal radius fracture ,best line of management ?	backslap of elbow
26	In a case of a mid-shaft humerus fracture, which nerve is most likely to be injured	Radial nerve
27	A 20-year-old male patient present to the ER after painful twisting of the ankle while playing football. Which of the following would be most informative regarding his diagnosis?	MRI
28	is not true about DDH	Hip X-ray is used for confirming the diagnosis of all age groups.
29	In fractures of metacarpal bones, the hand should be positioned in	The safe position
30	In patients with a brachial plexus lesion diagnosed with Erb's palsy, muscle is not affected	Interosseous muscle of the hand
31	In the case of a distal humerus fracture, what is the most important thing to assess	Distal pulse
32	A patient presents with inability to abduct his right shoulder. On examination abduction paradox and drops sign were positive. What is the diagnosis?	Complete rotator cuff tear
33	True regarding osteoblastoma	Possible malignant change risk
34	Best immobilization for a third metacarpal fracture	Safe immobilization
35	A patient is diagnosed with compartment syndrome. What is the best treatment?	fasciotomy
36	is true about chondroma	It's usually asymptomatic
37	False regarding perthe's disease	Onset > 10 years
38	A child presents with proximal tibial swelling and erythema. What is the most likely diagnosis?	Acute hematogenous osteomyelitis
39	A child falls and sustains a fracture of the lateral epicondyle. Which of the following complications is most likely to lead to cubitus valgus?	Malunion
40	On examination with legs straigt and the medial malleoli just touching, the distance between the knees was more than 10cm, DX	Genu varus
41	Regarding Monteggia fracture, is correct	Treat by restoring length of the ulna
42	complications is least likely to occur in a closed fracture	Infection
43	is a complication of scaphoid fracture	Avascular necrosis of the proximal 1⁄3
44	patient presents with a undisplaced fracture of the 3rd metacarpal bone. What is the best treatment?	Splint immobilization in safe position
45	is a complication of scaphoid fracture	Avascular necrosis of the proximal 1/3
46	is an important for fracture healing	Immobilization
47	a woman who presented to the ER with fall from height .on examination she was unconscious and had ankle swelling ,the initial step of management is skeletal survey ,which is comprised of what ?	Option C (could not safely map from original source) radiological assessment of knee, hip , spine and base of skull

#	Fact cue	Answer
48	a woman who presented to the ER with fall from height .on examination she was unconscious and had ankle swelling,the initial step of management is skeletal survey,which is compr...	radiological assessment of knee, hip , spine and base of skull
49	A 45-year-old man, who on examination had a sinus in his proximal thigh, was diagnosed with chronic osteomyelitis. Which of the following management is done for this patient?	All of the above (Options: A. Sequestrectomy; B. Antibiotics; C. Surgical debridement; D. All of the above; E. None of above)
50	A young patient presents with a painful shoulder joint, and there is a history of overuse due to repetitive activities. What is the most likely diagnosis?	Acute tendinitis
51	A young patient with a painful shoulder after over practicing has difficulty in elevating it. It is responding to conservative treatment. What's the possible diagnosis?	Acute calcific cuff tendinitis
52	The following is regarded as indications For CT scan in bone diseases except	Skeletal survey for detection of metastasis or multiple myeloma
53	What type of shoulder dislocation should be suspected after an epileptic seizure	Posterior shoulder dislocation
54	False regarding the menisci	It's hyaline cartilage
55	True regarding club foot	The treatment is usually non-operative
56	patient with painful wrist joint with positive Flinkelstein sign	dequarvain tenosynovitis
57	Talipes Calcaneovalgus differs from talipes equinovarus in all of the following except	The toes are touching the ground
58	A patient presents to the clinic with pain over the radial styloid and upon examination there is a positive Finkelstein sign. What is the most likely diagnosis?	De Quervain's Tenosynovitis
59	The earliest radiological feature to appear in acute osteomyelitis is	Periosteal reaction
60	A child presents with a red, swollen metaphysis and a high white blood cell count. What is the most likely diagnosis?	Acute hematogenous osteomyelitis
61	Simple bone cyst	Aspiration reveals a straw-coloured fluid
62	In patients with a brachial plexus lesion diagnosed with Erbs palsy, is not true	hand paralysis
63	A patient presents to the clinic with pain over the radial styloid and upon examination there is a positive Finkelstein sign. What is the most likely diagnosis?	De Quervain's Tenosynovitis
64	Non-union in closed fractures may due to	Inadequate immobilization
65	is true of rocker bottom foot	Talus is vertical
66	is not a feature of rocker bottom foot	Flat sole
67	A patient presents with an undisplaced distal radial fracture. What is the appropriate treatment?	Backslab below the elbow
68	A patient 50 yo with inability to lift the shoulder with positive abduction paradox test Dx	complete rotator cuff tear
69	A 25 year old patient presented with fracture of the pelvis after car accident, the principle of the treatment of this patient is:	Resuscitation of the patient should be hand in hand with clinical assessment
70	radiological abnormalities most accurately serves as a predictor of ankle syndesmosis disruption	' Tibiofibular clear space more than 6mm on AP
71	False regarding the denis weber classification of fractures	We rely on the level of tibial fractures
72	The most common complication of intertrochanteric fracture	Malunion

#	Fact cue	Answer
73	is true about Ivory exostosis	Affect all bones including skull
74	Osteoid Osteoma	Responds to NSAIDS well
75	The suspected shoulder dislocation after epilipsy is	posterior shoulder dislocation
76	A patient after the fracture presented with tense forearm and pain on stretching	compartement syndrome
77	Which movements are restricted in perthe's disease	Abduction and internal rotation
78	A patient falls backward on an outstretched hand and presents with a prominent acromion on shoulder plain radiograph. What is the diagnosis?	Anterior shoulder dislocation
79	What is a potential cause of non-union in bone fractures	Not immobilized completely
80	Talipes Calcaneovalgus differs from talipes equinovarus in all of the following except	The toes are touching the ground
81	Regarding Chondroma (Enchondroma), which answer is true	Usually asymptomatic
82	What is the most common dislocation of the hip	Posterior dislocation
83	is not true about fracture of calcanium	Results in avascular necrosis,
84	What nerve is commonly injured in anterior shoulder dislocation	Axilary
85	A 3 years old child presented with swelling and redness of proximal third of left tibia, his WBC :14000, ESR 47 wh could be the diagnosis	Acute hematogenous osteomyelitis
86	Proper immobilization includes	Above and below
87	False regarding the menisci	It's hyaline cartilage
88	A 3 yo child presented with crying, painful extended and pronated elbow, unable to flex it	pulled elbow
89	A 65 year old woman falls and sustains a severely displaced colle's fracture. What to do next?	Closed reduction and splint below the elbow
90	False regarding perthe's disease	Onset>10 years
91	A 25-year-old female athlete presents with twisting of ankle. She had a history of stress fracture. The attending doctor inquired about her nutrition and...	menstruation
92	A patient with tennis elbow, Tx	Physiotherapy
93	Which nerve can be injured with fracture of medial epicondyle of the humerus	Ulnar nerve injury
94	Concerning treatment of developmental dysplasia of the hip, statements is TRUE	Avascular necrosis of capital femoral epiphysis is common complication after open reduction
95	A patient after falling backward on stretching hand , on shoulder plain radiograph there is prominent acromion , Dx	anterior shoulder dislocation
96	A woman after an injury sustains a clavicular fracture that is more than 2 cm displaced and there is bulging of the skin. How to manage	Open reduction and internal fixation
97	' A 2.5-year-old female presented with painless limp of the left leg with limited abduction and positive Galeazzi test. X-Ray showed left DDH. is the recommended treatment? '	Open Reduction with Salter osteotomy
98	A 5-year-old boy with progress painless left limp. He had limited abduction. Which of the following is true regarding his condition?	Surgery during the fragmentation stage is most effective
99	is the most sensitive sign for DDH in a 6 month old child	Limited abduction in flexion

#	Fact cue	Answer
100	A 65-year-old with femoral neck fracture 6 weeks ago. What is the best treatment now?	Hemiarthroplasty
101	Best immobilization for a third metacarpal fracture	Safe immobilization
102	What is the complication of lateral condylar fracture if left capsized	Nonunion
103	A patient presents after a fracture with a tense forearm and pain on stretching. What condition is indicated?	Compartment syndrome
104	True regarding club foot	The treatment is usually non-operative
105	A child presents with a distal radius and ulna fracture. What type of cast is most appropriate for immobilization?	Above elbow cast
106	A 65 year old woman falls and sustains a severely displaced colle's fracture.	Closed reduction and splint below elbow
107	Most common fracture in children	Distal radius fracture
108	A 35-year-old patient presents with pain in the wrist joint, specifically on the radial side, which worsens with gripping or twisting movements. A positive Finkelstein test is noted. What is the most likely diagnosis?	De Quervain's tenosynovitis
109	True regarding osteoid osteoma	Responds well to NSAIDs
110	is not typically seen as a feature or complication of developmental dysplasia of the hip (DDH)	Avascular necrosis of the femoral head If not treated
111	A 45-year-old man, who on examination, he had a sinus in his proximal thigh. He was diagnosed with chronic osteomyelitis. Which of the following management is done for this patient?	all of above (Options: A. Sequestrectomy; B. antibiotics; C. surgical debridement; D. all of above; E. ORIF)
112	Which nerve is injured in anterior shoulder dislocation	Axillary nerve injury
113	A case who has developed tennis elbow. What's the appropriate management?	Physiotherapy
114	the complication which is not seen in closed fracture is	infection
115	is not an ankle ligament	Posterior Cruciate Ligament (PCL)
116	A case was given, as was the diagnosis. Which of the following is the treatment for cubitus varus?	Wedge osteotomy with the base located laterally
117	patient presents with a minimally displaced fracture of the clavicle.What is the best treatment?	Arm sling and analgesia
118	A 20 year old male sustained an injury a while ago and got a fracture. He was treated by an osteopath. He now has cubitus valgus and has pain and tingling over the medial one and a half fingers. How do you treat it?	Anterior transfer of ulnar nerve
119	On examination with legs straigt and the medial malleoli just touching, the distance between the knees was more than 10cm, Dx	Genu varum
120	A patient presented with interartucular displacement Tx.	Open reduction and internal fixation (ORIF)
121	is not a feature of the talus bone	It has anterograde blood flow
122	The complication of lateral condylar fracture if left capsized is	non union
123	is the treatment for cubitus varus	Wedge osteotomy with the base located laterally
124	A Skeletal survey is indicated in any patient with	Unconsciousness
125	A child presents with a distal radius and ulna fracture. What type of cast is most appropriate for immobilization?	Above elbow cast
126	True regarding osteosarcoma	Sunburst appearance is seen in affected areas
127	After reduction of anterior shoulder dislocation, the patient should be instructed to	Immobilization of the shoulder in a sling for about 3 weeks

#	Fact cue	Answer
128	The presence of respectful, mindful, and heedful interactions among all parties is one of the characteristics of work relationships associated with successful practices	True
129	A 56-year-old woman sustained a humorous fracture, injuring her radial nerve. Which of the following findings would be expected on electromyography (EMG)/nerve conduction studies (NCS)?	Conduction block
130	If A 75 year old woman presented after simple fall on the ground with severe pain in the left hip and inability to walk. On examination; the left leg is shorter then right leg a...	Intertrochanteric fractured femur
131	When you receive a 55 year old female with history of breast cancer and fracture right femoral shaft after simple fall on the ground. What do you think the type of such fracture	Pathological fracture

Urology (99 questions) (99 questions)

#	Fact cue	Answer
1	is the gold standard investigation for a renal mass	CT with or without contrast
2	A 65-year-old woman with a 40 pack-year smoking history had intermittent hematuria for 1 year. Her urine cytology was initially negative, and over the past 6 months, urine cytology was positive. Cystoscopy by urology revealed a fungating mass. Biopsy was positive for urothelial cancer, but deep muscle was not present. Physical examination revealed obesity and hypertension, no jaundice, and no hepatosplenomegaly. Labs revealed anemia with hemoglobin of 10.6 g/dL and creatinine of 1.3 mg/dL. What is the most appropriate next diagnostic step?	Repeat cystoscopy with transurethral resection of bladder tumor { TURBT }
3	class of drug is not used in BPH	Phosphodiesterase stimulator
4	differentiates urinary retention from obstructive anuria	Urge to void
5	Which is wrong regarding the indication of admission in renal stones	Pain for more than half an hour
6	statement are not true regarding epidemiology of VUR	Three-quarters of children being treated for reflux are Boys
7	is not a complication of BPH	Renal stone
8	Which one is functional unit of lower urinary tract	detruser muscle
9	A patient presented to Emergency Department with chest pain, he has history of taking viagra before. What's the management?	Do not give nitrates in the next 72 hours.
10	post renal anuria causes	Recent hysterectomy
11	A case of LUTS with a suprapubic mass and HBA1C, which investigation is used?	Urodynamic study
12	True regarding bilharziasis	It is due to egg in subepithelial layer of bladder
13	Regarding the epidemiology of stones Which statement is incorrect	More common in white females
14	Patient with BPH has IPSS of 19 means	moderate disease
15	A couple presents with primary infertility of 2 years. is true	Both male and female partner should be assessed
16	Which statement is true regarding hydatid disease	It is always treated surgically
17	Mode of genetic inheritance in BPH	Autosomal dominant
18	Which one is not an indication for admission	People who fail to respond to analgesia after 1/2 hour.

#	Fact cue	Answer
19	A 25-year-old man has history of sexual contact 2 weeks ago, he has symptoms of fever rigor and suprapubic pain and unable to void	acute prostiotis
20	A 71 year old male patient with 80 year-pack history of smoking in the last 30 years presents with gross hematuria and irritative LUTs syndrome. Which of the following is the correct diagnosis?	Urinary bladder urothelial carcinoma
21	Which one is emergency management of Pyonephrosis	Percutaneous nephrostomy
22	33.all of the following causes acute urinary retentions EXCEPT	Acute cystitis
23	is true about genitourinary tuberculosis	It initially affects the kidneys and prostate
24	Male with primary infertility and azoospermia	HX of Bilateral inguinal surgery
25	chronic urinary retention	Flaccid neurogenic bladder
26	Which receptor found in prostate and bladder neck	– Alfa 1a receptor
27	All of the following are differentials for left ureteric colic in a female patient except	Acute appendisities
28	A VUR case with grade I VUR. is not correct regarding the management	Antimicrobial drugs, followed by Chronic suppressive therapy should be continued for 6 months or more
29	A 25 year old male with primary infertility, normal sperm parameters.	Female factor infertility
30	the most common subtype of renal cell carcinoma RCC	clear cell carcinoma
31	Following surgery for varicocelectomy, how long does it take for SFA parameters to improve	3 months
32	is true about watchful waiting	None of the above (Options: A. For good functioning patients; B. For high risk cases; C. Intervention once progression occurs; D. Requires frequent testing or regular biopsy; E. None of the above)
33	A 63 year old patient with hematuria, anemia, pyrexia, and hypertension. Which of the following is the correct diagnosis?	Renal cell carcinoma
34	A man has a 4 mm stone at the ureteropelvic junction. is true	It can pass spontaneously with hydration
35	A pregnant lady presented with abdominal pain and fever, on examination she had renal angle tenderness and pylonephritis on U/S. What's the next step in management	Urinalysis and urine culture
36	which one is not true regarding stages of VUR	stage III there is clubing clayx
37	does not occur in acute urinary retention	overflow incontinence
38	Male with primary infertility and abnormal sperm parameters, what's the dx? (Cross Matching)	Left varicocele
39	A case of renal trauma with 2 cm laceration and small perinephric hematoma and no leak of contrast, what's the management?	Conservative management and follow up
40	A male patient who is a known case of diabetes, his HBA1C was 14%, which Inx is useful?	Urodynamic study
41	A male patient presented with trauma to their left loin. He had hematuria and complained of loin pain and ecchymosis on their left side. Which one of the following is the best investigation for diagnosis?	CT-Scan with contrast
42	Carcinogenesis of bilharziasis (squamous cell carcinoma of the bladder) is related to	Local mechanical irritation by the eggs
43	A case of an old male with dribbling urine, frequency, urgency, and a large suprapubic mass:	Chronic urinary retention due to BPH

#	Fact cue	Answer
44	What is the gold standard imaging modality for evaluating a renal mass	CT scan
45	True regarding renal cell carcinoma	it can occur in von hippel-lindau disease
46	drugs inhibit 5-alpha reductase type 2 selectively	Finasteride
47	A 4 year child presents with recurrent UTI. is true	Voiding cystourethrogram is the gold standard for diagnosis
48	A 70 years old male presents with 3 years of erectile dysfunction, Second step	Send for Hba1C We have to exclude diabetes as it is the most common cause of erectile dysfunction.
49	Case with VUR, mild-moderate dilation of ureter and pelvicalyceal system. What grade is this?	Grade III
50	A young person with urinary retention and a history of sexual contact, which investigation is used?What is the most appropriate investigation?	Retrograde urethrogram
51	True regarding a vur case	The offspring of an affected parent has up to 50%
52	What is the most common type of renal cell carcinoma (RCC)	Clear cell carcinoma
53	A 25 year old a er a car injury presents with gross hematuria, mild difficulty with urination, suprapubic bruising. Which of the following is the diagnosis?	extraperitoneal laceration
54	A 15-year-old boy presented to the clinic after trauma to the perineum while playing football. He had blood at the urethral meatus and scrotal ecchymosis. Which of the following is the most likely diagnosis?	Bulbar urethral injury
55	A 50 years old male diagnosed as BPH is taking tamsulosin and finasteride. On follow up evaluation, his PSA was 3.2 ng/ml. What's the management	Follow up yearly
56	Which one isn't a surgical indication for BPH	Urinary retention
57	A patient presents with sudden onset of painful, red, swollen testicles. What is the most likely diagnosis?	Testicular torsion
58	statements is wrong regarding renal cell carcinoma (RCC)	It is always treated by radical nephrectomy
59	Male with primary infertility and moderate secondary sexual characteristics abnormality:(cross matching)	Hormonal Cause
60	Not a complication of hormonal therapy for prostate cancer	Hypoglycemia
61	is true	)The presence of dysmorphic RBCs may indicate the presence of glomerular disease.
62	is not an established risk factor for RCC	Diabetes
63	A 25-year-old male patient presented with primary infertility. He had recurrent testicular pain. On investigation he had oligospermia with a normal FSH level. Which of the following could be the cause?	The cause is most likely varicocele
64	All of the following are true in evaluating a patient of urinary retention, except	Fever + Rigor (pyelonephritis)
65	A 12 years old boy presented to ER with inability to void after playing football, He has suprapubic pain and palpable mass. What's the best approach to reach the diagnosis	Retrograde urethrogram
66	-year-old male patient presented with primary infertility. On investigation he had azoospermia with a normal FSH level. could be the cause	Normal FSH level due to obstruction
67	alpha-blockers used in BPH is uroselective	Alfuzosin

#	Fact cue	Answer
68	True regarding a case with pyospermia	All of the above (Options: A. Pyospermia (or leukocytospermia) may be due to either a genital tract infection or an inflammatory immunologic response.; B. Cultures to rule out urinary and genital tract infection should be performed.; C. The etiology may never be determined and pyospermia can resolve without specific treatment and ROS are produced may self limited.; D. A & B only; E. All of the above)
69	Which is the most common renal cancer	Renal cell carcinoma (RCC)
70	Regarding pyonephrosis is correct	It follows obstructive uropathy
71	is the most common finding on physical examination in male infertility	left varicocele
72	A patient with chronic prostatitis is most likely to develop which of the following complications?	Epididymitis
73	Acute urinary retention which one is related to it	Acute suprapubic pain with fever and inability to void
74	A young adult presents with a firm, non-tender, solid mass in the scrotum that does not transilluminate. What is the most likely diagnosis?	Seminoma
75	A 22 year old male with azoospermia and lack of secondary sexual characteristics.	Hypogonadism can be the cause
76	A case of staghorn stone, what is it made of and which organism causes it?	Struvite, Proteus
77	UPJ case, is not correct	Often bilateral
78	Which one done for diagnosis of acute pyelonephritis	Urine culture and sensitivity
79	all of the follwoing are can regard as differential diagnosis of left ueretric colic except	Acute appendicitis
80	45 year old male with secondary infertility, now has azoospermia	Serum FSH , LH, and testosterone
81	In evaluation of 25 years old man with primary infertility for 2 year and azoospermia . blood test for FSH hormone done for him reveal normal level	normal level of FSH mostly reveal obstructive azoospermia
82	Mr hasan woke up at 8 am felt like he need to pass urine, but he couldn't start, after 2 hr pain start in lower abdomen he felt as if he was going to explode so he come to emerg...	False
83	60-year male patient present to emergency department with involuntary loss of urination (drippling), feeling incomplete bladder empty, no pain, no fever, no hx of stone,past me...	Neurogenic bladder
84	Intraperitoneal bladder perforation can be treated conservatively	False
85	30 years male patient present to emergency department with 6hr inability to urinate, had suprapubic pain 2 day ago had fever, malaise LUT symptoms, had past history of sexual co...	Acute urinary retention due to active prostatitis
86	65 years male patient present with red color urine, rt loin pain, no fever, no lut sx, no history of trauma, no history of stone, he deny eating any beet or blackberry, any rece...	Ct-scan of abdomin and pelvis native film
87	Retrograde cystography is diagnostic test of bladder injury	True
88	which point is wrong about urothelial bladder cancer	Cigarette smoking depend on duration and but not intensity of smoking
89	Is retrograde ureterography is diagnostic for ureteric injury	True

#	Fact cue	Answer
90	tadalafil 5mg originally used for erectile dysfunction and pulmonary hypertension, it has been found that also aid in management BPH	True
91	in belhariziasis of the urinary bladder the main pathological changes are caused by the presence of worms in the wall of urinary bladder	False
92	operations is not be complicated with ureteric ligation	Radical gastrectomy
93	70 year old male patient present with LUTS , DRE non remarkable , PSA 2.5 ng/ml , TRUS prostate size 150 , bladder capacity 350 cc , Post-voiding residual 100 cc, s creatinine 3mg/dl best treatment option:	Open prostatectomy
94	all the following medications can cause urinary retention except	Ciprofloxacin
95	false about BPH	prostate size correlate with severity of symptoms or degree of obstruction
96	all the following true about bladder Cancer except	Most common presentation painful gross hematuria
97	if patient suspected to have urethral injury can we insert urethral catheter	False
98	30-year male patient present to emergency department with 6hr inability to urinate, had suparpubic pain,had past history of sexual contact 1 year ago with urethral discharge,no...	Retrograde urethrogram
99	The large majority of patients with early-stage CaP are symptomatic true or false	False